Biotechnology

CAMP4 Therapeutics Corporation (CAMP)

RATING	PRICE TARGET	PRICE	52-WEEK RANGE
BUY	$8.00	$3.43	$2.86 - $7.75

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Summit Push Begins in SYNGAP1 - Initiating Coverage with a Buy and $8 PT

We are initiating coverage of CAMP4 Therapeutics (CAMP) with a Buy rating and an $8 price target. Lead asset CMP-002 is an antisense oligonucleotide (ASO) in Ph1/2 development for SYNGAP1-related disorder, a rare genetic neurological disorder characterized by moderate-to-severe intellectual disability, early childhood developmental delays, and epilepsy. SYNGAP1 affects ~11K patients in the US and there are no approved drugs. We believe CMP-002 has the potential to be the 1st disease-modifying therapy for SYNGAP1 and model ~$2.1B in non-risk-adjusted peak sales. We conducted a proprietary survey of US-based SYNGAP1-treating physicians that highlights the unmet need, market opportunity, and enthusiasm for a therapeutic option with a CMP-002-like profile, all of which we view as a positive for CAMP. Please see note for full survey.

- **SYNGAP1 is a rare genetic neurological disorder that affects ~11K patients in the US - there are no approved drugs or effective therapies.** SYNGAP1 is a haploinsufficiency - a mutation in 1 copy of the SYNGAP1 gene results in suboptimal 50% SYNGAP1 protein production. SYNGAP1 protein is a critical regulator of proper neuronal signaling & function, and insufficient levels (i.e. in SYNGAP1 patients) result in a multitude of severe symptoms, including neurological (seizures/epilepsy), developmental delays (cognitive, speech), and behavioral problems. We estimate SYNGAP1 prevalence is ~11K in the US (and ~10K in EU4+UK), of which we estimate ~15% (~1.5K) are diagnosed. There are no approved drugs and SoC is suboptimal symptom management, typically with anti-seizure medications.

- **CMP-002 has the potential to be the first disease-modifying therapy for SYNGAP1 - we model unadjusted WW peak revenue of $2.1B for CMP-002 in SYNGAP1.** CMP-002 is an intrathecally-administered ASO designed to upregulate SYNGAP1 expression and restore protein to normal "healthy" levels. Given the MoA, we believe CMP-002 has disease-modifying potential. A Ph1/2 MAD trial evaluating CMP-002 in SYNGAP1 patients is expected to start in 4Q26, and our base case assumes initial results in 1H28.

- **We conducted a proprietary survey of 16 US-based SYNGAP1-treating physicians.** The collective results suggest there is: 1) a high unmet need for effective therapies that treat the neurological and developmental symptoms of SYNGAP1; & 2) high enthusiasm and a strong commercial opportunity for a therapy with a potential CMP-002-like profile. ***Full survey results are below.***

- **Initiating coverage with a Buy rating and $8 PT.** Our valuation and PT are based on a blend of two valuation methods: 1) a DCF analysis through 2050 using risk-adjusted revenues, an 9% WACC, and 0% terminal growth; & 2) an EV/peak revenue multiple analysis using a 7.0x peak revenue multiple and risk-adjusted revenues. Our model and $8 PT assigns $6/share for CMP-002 in SYNGAP1 and $2/share for net cash.

KEY DATA

Market Cap (MM)	**$215.2**
Price (10/13/2026)	$3.43
52-Week Range	$2.86 - $7.75
Shares Outstanding	62.75
Avg. Daily Volume	317,436.5

ESTIMATES

FY (Dec)	2025A	2026E	2027E
EPS			
Q1	(0.62)A	(0.32)A	-
Q2	(0.62)A	(0.26)A	-
Q3	(0.55)A	(0.28)E	-
Q4	(0.86)A	(0.28)E	-
Year	(2.65)A	(1.13)E	(1.04)E
Growth		NM	NM
P/E	NM	NM	NM
FY (Dec)	**2025A**	**2026E**	**2027E**
Rev. (MM)($)			
Q1	0.9A	1.3A	-
Q2	1.5A	0.7A	-
Q3	0.8A	0.7E	-
Q4	0.3A	0.7E	-
Year	3.5A	3.4E	0.0E
Growth Rev		(2.9)%	(100.0)%

At a Glance

OUR INVESTMENT THESIS

We believe the current market value of CAMP does not adequately capture the value of the risk-adjusted revenues of the CMP-002 SYNGAP1 program. We believe CMP-002 has the potential to be a best-in-class and disease-modifying therapy for SYNGAP1, which we believe holds significant value. We think the stock will be driven primarily by a positive clinical developments and data from the SYNGAP1 program.

BULL CASE ASSUMPTIONS

- Initial Ph1/2 cohort data comes before 1H28 - CMP-002 shows strongly positive signals from the Ph1/2 initial cohorts in SYNGAP1 (e.g significant seizure reduction and/or improvements in non-seizure measures like motor & gait, communication, behavior, development & cognition)
- A clear and well-defined development path forward and strong alignment with regulators emerges quickly based on very strong initial CMP-002 Ph1/2 data in SYNGAP1
- Positive POC data in SYNGAP1 helps de-risk and validate the platform and is a positive readthrough to additional haploinsufficient / loss-of-function high-value rare disease indications, including the SHANK3 program
- Upward revisions of sales estimates to >$2.5B unadjusted peak sales of CMP-002 in SYNGAP1
- $16 price target

BASE CASE ASSUMPTIONS

- CMP-002 Ph1/2 data tracks to 1H28 and shows trends in improvement across key efficacy measures (e.g. seizure reduction, etc.)
- The totality of the CMP-002 Ph1/2 data suggest a viable development path forward directly into a pivotal/ registrational trial in SYNGAP1 with good regulator alignment
- Data from the initial Ph1/2 cohorts of CMP-002 in SYNGAP1 shows the potential for an attractive dosing profile
- CMP-002 dosing beyond 3 months is cleared by regulators
- DC nomination from SHANK3 program announced in 2027 helps fill out pipeline

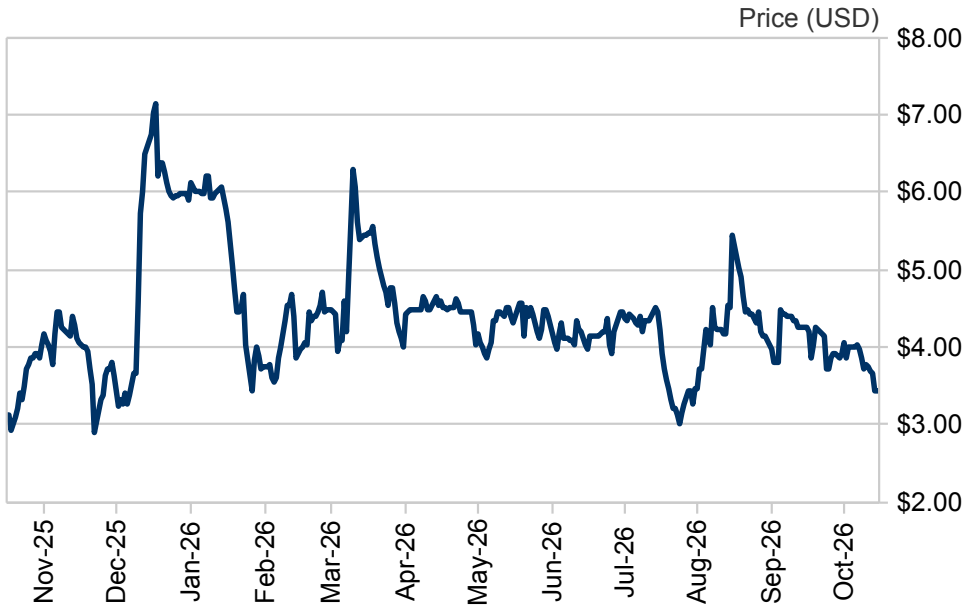
BEAR CASE ASSUMPTIONS

- CMP-002 Ph1/2 timeline or enrollment delays, initial data readout pushed out beyond 1H28
- Negative safety signals or ambiguous efficacy readings from the initial Ph1/2 cohorts of CMP-002 in SYNGAP1 could put significant pressure on the stock
- Lack of clarity and visibility on a potential registrational path forward (trial design, endpoint selection, etc.) for CMP-002 in SYNGAP1
- Strongly positive data from competitor SYNGAP1 programs (e.g. PRAX, STOK/ACAD, Quiver, GondolaBio) sets a high bar for success and puts competitive pressure on CAMP
- CAMP requires significant dilutive financing
- $1 price target

COMPANY DESCRIPTION

CAMP is biotechnology company focused on the discovery and development of RNA-targeting therapeutics, with the goal of upregulating gene expression and restoring healthy protein levels to treat a range of genetic diseases. The company was founded in 2016, completed an IPO in October 2024 and is headquartered in Watertown, MA.

PRICE PERFORMANCE



KEY DATA

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PIPELINE

Product	Description	Indication	PC	Ph1/2	Ph3	Review	Market	Commercial Rights	Comments
CMP-002	antisense oligonucleotide (ASO) targeting *SYNGAP1*	SYNGAP1-related disorders		•				CAMP4	-
TBD	antisense oligonucleotide (ASO) targeting *SHANK3*	Phelan-McDermid Syndrome	•					CAMP4	-
CMP-001	antisense oligonucleotide (ASO) targeting *CPS1*	Urea Cycle Disorders		•				CAMP4	Seeking partner

CATALYSTS

High	± >10%	Positive
Medium	± 5-10%	Neutral
Low	± 3%	Negative

Timing	Product	Indication	Event	What to look for?	Stock Impact	Needham View	Stock Readthroughs
4Q26	CMP-002	SYNGAP1-related disorder	Ph1 SAD/MAD trial start	-	Medium	-	STOK, ACAD, PRAX
2027	TBD	Phelan-McDermid Syndrome	DC nomination	-	Low		-
1H28	CMP-002	SYNGAP1-related disorder	Initial results Ph1 SAD/MAD trial	-	High		STOK, ACAD, PRAX

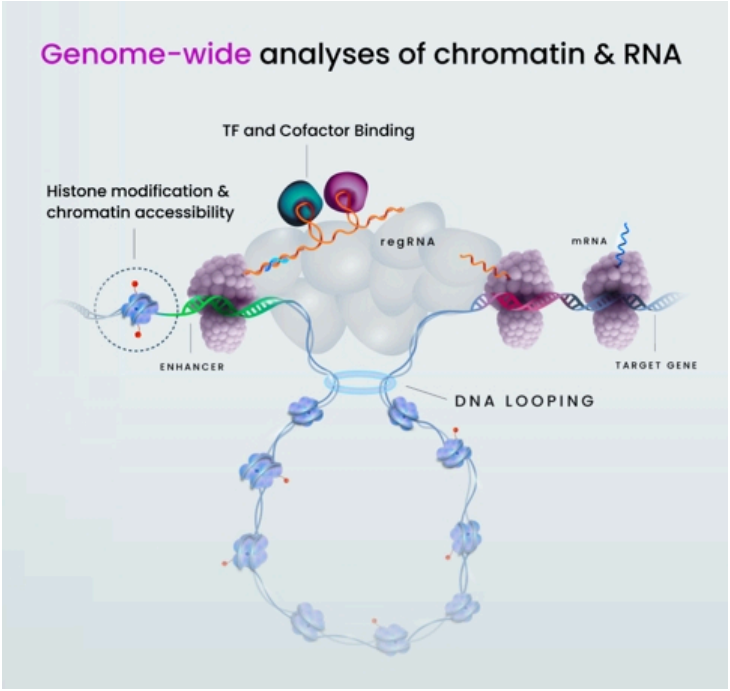
CAMP4 PLATFORM BACKGROUND

CAMP Background and the RAP Platform: ASO-mediated gene upregulation

CAMP's RAP platform utilizes ASOs to upregulate an endogenous gene without altering DNA. The approach works through regulatory RNA (regRNAs), a class of RNA that play an important role in controlling gene expression by acting locally on transcription factors at the enhancer/promoter level. CAMP maps the regRNA that is controlling a target gene of interest and designs an ASO that binds to the regRNA, which amplifies gene expression. Mechanistically, the ASO binds to the target regRNA and displaces transcriptional repressor proteins, which drives more transcription and ultimately the production of more protein.

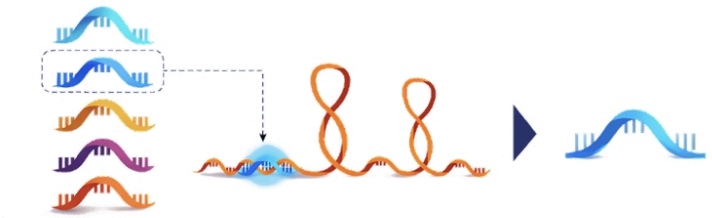
regRNAs are short-lived and relatively scarce, so CAMP built a proprietary regRNA capture method to identify them and has assembled a proprietary catalog of regRNA sequences across diverse cell and tissue types that could be amenable to therapeutic intervention. The use of ASOs is a clinically-validated therapeutic modality, with established chemical composition and delivery methods. CAMP's approach to target regRNAs allow for a programmable, tunable, and re-dosable way to upregulate a protein of interest, while avoiding the drawbacks and issues of delivery, immunogenicity, and safety associated with certain gene therapy or gene editing approaches.

CAMP's RAP Platform Step 1 - Using genome-wide analysis of chromatin & RNA for identification and mapping of regRNA candidates



Source: CAMP presentation Sep 2026

CAMP's RAP Platform Step 2 - regRNA screening to identify lead ASO drug candidates



CAMP's RAP Platform Step 3 - Optimization of ASO drug candidates for activity, PK/PD, and safety



Source: CAMP presentation Sep 2026

SYNGAP1 BACKGROUND, UNMET NEED, AND CAMP'S LEAD DRUG CMP-002

SYNGAP1 Background

SYNGAP1-related disorder (abbr. SYNGAP1) is a monogenic developmental and epileptic encephalopathy (DEE) caused by haploinsufficiency of the SYNGAP1 gene (one mutated SYNGAP1 gene and one unmutated functioning copy), which yields SYNGAP1 protein ~50% of normal levels. The large majority are de novo mutations, with truncating (loss-of-function) variants predominating.

> **Epidemiology:** Prevalence/Incidence: CAMP estimates the prevalence is ~11K in the US and ~10K in the EU4 + UK. Based on a literature review using incidence rates and adjusted for population, we arrive at similar estimates that are in line with CAMP. For example, prevalence and incidence rates in the literature vary, but generally range from 1-4 per 10K; or ~6 per 100K births. SYNGAP1 is estimated to represent 0.5-1% of all US cases of intellectual and development disability in children (some sources

cite up to 2%). Applying that to the estimated US intellectual/development disability cases of 7-8.5M, and adjusting for the US population ≤18 y/o, gets a ~12K US prevalence. We provide our detailed model and assumptions below, and references for the assumptions are in our model and available upon request.

Identified patient estimates: CAMP estimates that diagnosis rates are ~15-20%. As of 9/30/26, the CURE SYNGAP1 global census counted 1,871 diagnosed patients worldwide. This is the total known to SYNGAP1 advocacy organizations and family support groups, and is explicitly not a registry, and it also excludes benign/likely-benign variants and VUS without an accompanying clinical diagnosis. The census added +65 patients in 3Q26 (following +54 in 2Q26) and is adding ~200 new patients per year. In a 2023 R&D Day presentation, Praxis (PRAX; Buy; Fadia) estimated there are ~2K diagnosed SYNGAP1 patients in the US based on a US claims database analysis (presentation here).

Key Sources for SYNGAP1 Patient Characterization



Source: CAMP presentation Sep 2025

Symptoms: The most common symptoms are (% of estimated patients occurring): developmental delay / intellectual disability (100%); generalized epilepsy (~84%); and autism and other behavioral abnormalities (<50%); other common symptoms include hypotonia, sleep disturbances, and feeding issues. Developmental delay precedes seizure onset in the vast majority (~95%) of patients, and initial symptoms typically start in infancy. Seizure onset then follows at a median age of ~3 years. No robust genotype–phenotype correlations have been established.

Diagnosis: A genetic test is required to confirm a diagnosis. Because early symptoms (developmental delay, hypotonia, autism) overlap with other neurodevelopmental disorders, the journey from onset to diagnosis often spans several years. The median age of genetic diagnosis is ~3.8 years (IQR 2.7-6.8; Citizen Health + CHOP natural-history cohorts), consistent with the ~4-year median (range 2-7) in Wiltrout 2025. ***Diagnosis rates remain low - we model ~15-20% based on our literature/public-resource review, and the ~1,871 census against prevalence implies the global rate may be even lower.*** SYNGAP1 is not included in standard newborn screening panels, and most newborns are not screened for genetic disease at birth, so like most rare monogenic disorders the majority of cases go unidentified.

Treatment: There are no approved or disease-modifying therapies, and no there is nothing on the market that addresses the underlying genetic cause. Since most patients have epilepsy, anti-seizure medications are used off-label as front-line therapy (e.g. Epidiolex, valproate, clobazam, levetiracetam, and lamotrigine are most commonly used), and polypharmacy is common. However, up to 50% of SYNGAP-1 related epilepsies are drug-resistant. In drug-resistant cases, patients may undergo vagus nerve stimulation or a strict ketogenic diet. Melatonin and perampanel are used in some patients with sleep disturbances, and behavioral and psychiatric drugs (e.g. risperidone, aripiprazole) are used in some patients to help manage the symptoms of autism and behavioral issues.

Clinical endpoints: There are no approved or regulatory validated endpoints for SYNGAP1. In the Phase 1/2 trial, CAMP plans to evaluate a range of relevant measures including seizures, Bayley-4, Vineland-3, Aberrant Behavior Checklist, 2nd edition (ABC-2), Observer-Reported Communication Ability Measure (ORCA), Gross Motor Function Measure (GMFM), clinician global impression (CGI-S/CGI-I), and patient-reported outcome for sleep, behavior, and quality of life (QoL).

WHAT ARE THE UNMET NEEDS IN SYNGAP1-RELATED DISORDER?

Improved diagnosis: The average time to diagnosis is ~5 years from the first symptom. Diagnosis rates are relatively low and are estimated to be ~15-20% of the total prevalence. Improvements in time to diagnosis and diagnosis rates are needed, and we feel that these would likely dramatically improve if a disease-modifying therapy were available.

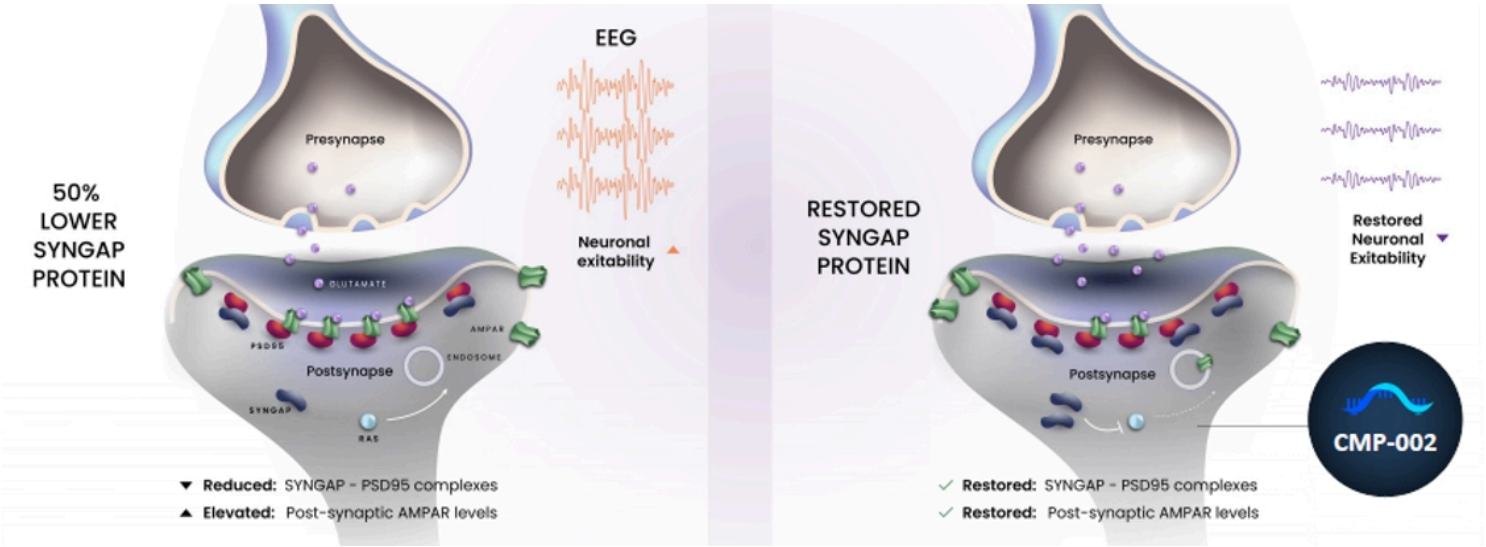
Better seizure control that anti-seizure medications (ASMs) can't deliver: ~85% of patients have epilepsy, and these patients often present with multiple types of seizures that are commonly resistant to the standard regimen of ASMs. Seizure burden/control is one of the most important areas of unmet need in SYNGAP1.

No disease-modifying therapies available: There are no approved drugs for SYNGAP1. SoC drugs are used to manage symptoms and include off-label use of anti-seizure medications, cannabinoids, and sleep medications, but none of these options address the genetic cause (haploinsufficiency) of the disease. CMP-002 is being positioned as the first disease-modifying therapy.

Therapies that improve cognition / intellectual disability: ~100% of SYNGAP1 patients have an intellectual disability, and there is currently no therapy that improves this trajectory. SoC in this area is limited to behavorial/educational/speech therapy. Along with seizures, this is one of the most important areas of unmet need.

CMP-002 is an antisense oligonucleotide (ASO) designed to bind to a specific regulatory RNA sequence, thereby increasing SYNGAP1 gene expression and protein production. By restoring these protein levels closer to wild-type, the therapy ultimately aims to normalize synaptic function to improve symptoms and potentially modify the disease in patients with SYNGAP1-related disorders.

Figure 4 - CMP-002 Mechanism of Action: designed to increase production of the SYNGAP1 protein, thereby normalizing synaptic function in patients with SYNGAP1-related disorder



Source: CAMP presentation Sep 2026

CMP-002: PRECLINICAL DATA SUMMARY

CMP-002 shows target engagement and proof-of-mechanism in human cells and translation in a "humanized" SYNGAP1 mouse model (Figure 5)

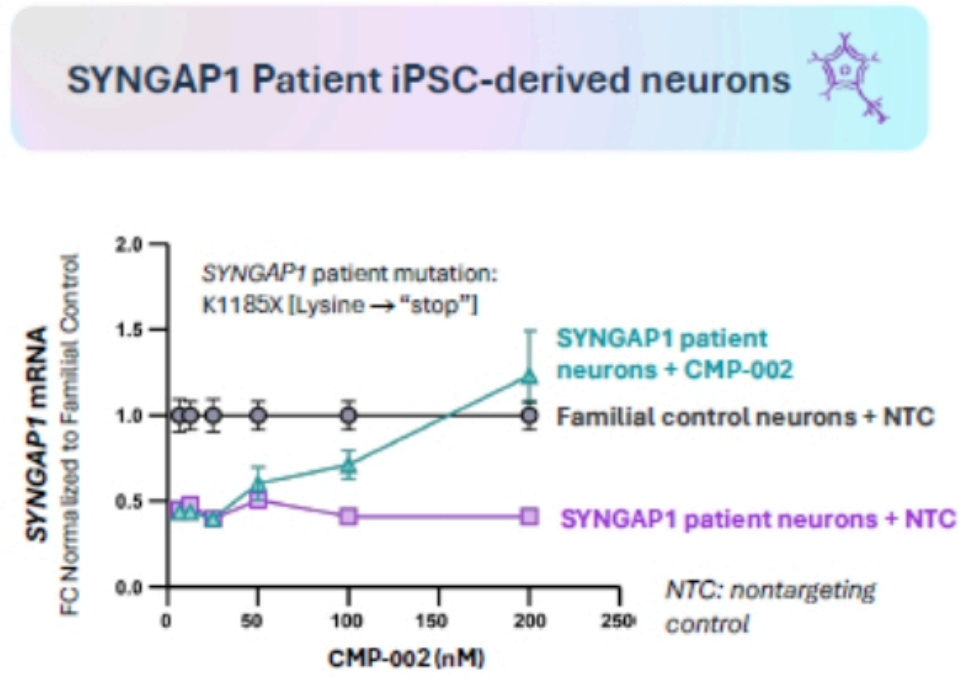
Figure 5 shows the data for an *in vitro* experiment in SYNGAP1 patient iPSC-derived neurons. The graph plots the effect of increasing CMP-002 concentrations (0 to 200 nM) or a nontargeting ASO control ("NTC") on SYNGAP1 mRNA levels. The three curves represent: 1) a familial control + NTC (grey line); 2) SYNGAP1 patient neurons + NTC (purple line); and 3) SYNGAP1 patient neurons + CMP-002 (teal line). The data are normalized to the familial control neurons + NTC (grey line), so SYNGAP1 mRNA levels are displayed as x-fold in reference to the control.

Since the familial (wild-type) control (grey line) represents the wild-type SYNGAP1 mRNA levels, by definition it stays at 1.0x across the entire dose range. In the SYNGAP1 patient neurons + NTC (purple line), the SYNGAP1 mRNA levels remain ~50% below the control (wild-type levels), and represents the expected haploinsufficiency (~50% expression levels) baseline. In the SYNGAP1 patient neurons + CMP-002

data (teal line), the SYNGAP1 mRNA increases in a dose-dependent fashion, starting from ~0.5x at concentrations <50 nM, and increasing to ~1.25x at 200 nM.

> **Needham takeaways:** the data shows that CMP-002 is able to increase SYNGAP1 mRNA levels from ~50% of normal to normal healthy control levels in a dose-dependent fashion. Given the lack of impact of the NTC control on SYNGAP1 mRNA expression levels, the data suggests that CMP-002 is engaging the target and producing the desired effect and is not due to an off-target mechanism or artifact.

Figure 5 - CMP-002 Dosing Leads to Functional Efficacy in a SYNGAP1 Haploinsufficient Mouse Model



Source: CAMP presentation Sep 2026

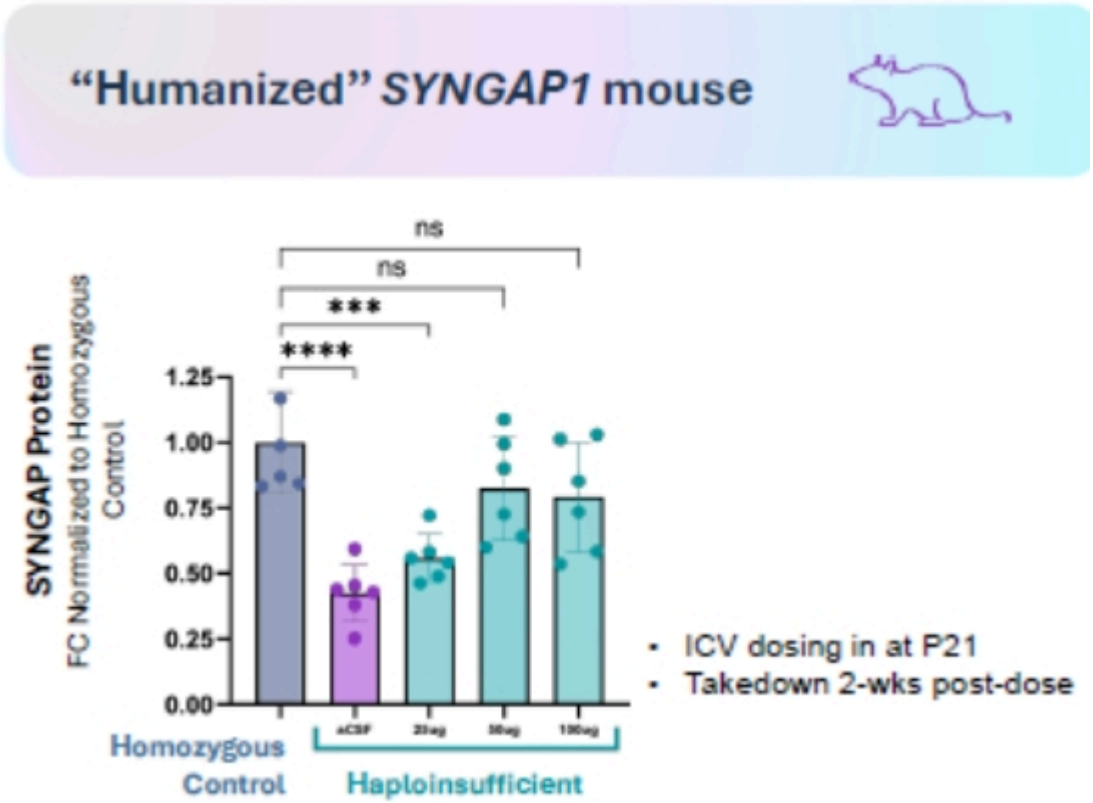
CMP-002 Data in Humanized Mouse Model Restores SYNGAP protein to near wild-type levels. (Figure 6)

In a separate set of *in vivo* experiments, CAMP evaluated CMP-002 in a humanized SYNGAP1 mouse model, to determine whether SYNGAP1 protein levels could be increased with treatment. The data below show ICV dosing at P21 in a humanized SYNGAP1 mouse model and the data readout is SYNGAP1 protein, normalized to a homozygous control.

The figure shows *in vivo* data from a humanized mouse showing SYNGAP1 protein levels normalized to a homozygous control (grey bar). The bars to the right of the control show data in haploinsufficient mice, with the artificial CSF (aCSF) control (purple bar) showing the expected ~50% SYNGAP1 protein expression relative to the homozygous control (grey). Across the three ascending doses (25 / 50 / 100 ug) of CMP-002 in haploinsufficient mice, the SYNGAP1 protein levels start at ~50% and increase in a dose-dependent fashion to ~75% of the homozygous control group (which approximates the wild-type levels).

> **Needham takeaways:** We note there are the usual preclinical caveats to the data, including different readouts across data sets (*in vitro* mRNA levels vs. protein levels in a mouse), so not a direct comparison within a single system. That said, we think the takeaways are that CMP-002 increased SYNGAP1 protein to wild-type levels in a dose-dependent manner, and the data suggests that CMP-002 target engagement/activity *in vitro* translates to the transcription level (mRNA) and converts to functional protein *in vivo*. We note that in the mouse model, the magnitude of SYNGAP1 protein expression is roughly calibrated to the therapeutically relevant window, in that the dose levels shown restore SYNGAP1 protein to levels above the haploinsufficent baseline but do not exceed the wild-type levels, which could be problematic from a safety/toxicity perspective.

Figure 6 - CMP-002 Data in Humanized Mouse Model - restores SYNGAP1 protein to near wild-type levels



Source: CAMP presentation Sep 2026

Intrathecal administration of CMP-002 in NHPs leads to broad, dose-dependent distribution across disease-relevant brain regions and a ~1.3-1.6x increase in SYNGAP1 protein levels. (Figure 7)

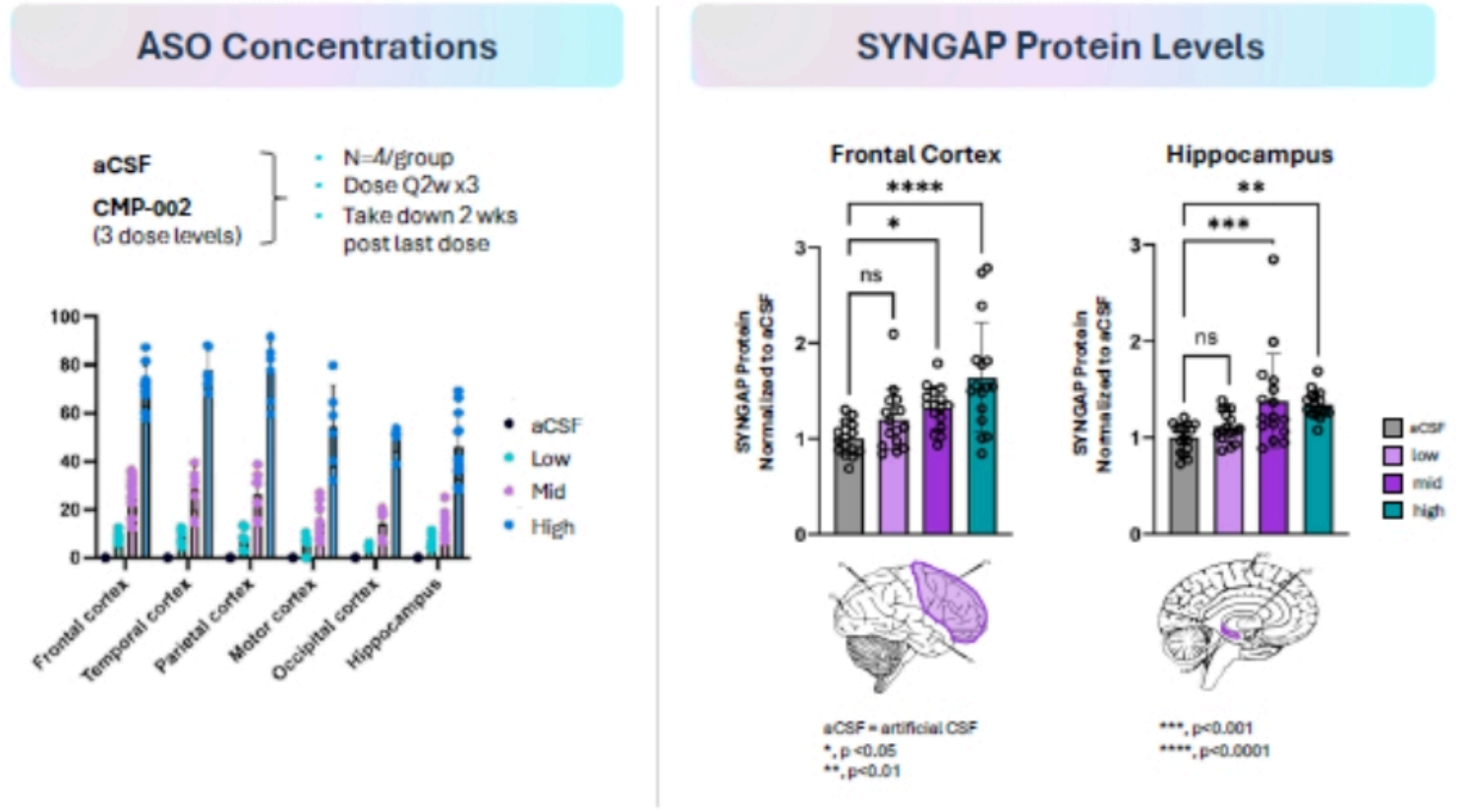
Study design: CMP-002 was administered via intrathecal injection (low/mid/high dose) Q2W x 3 doses to healthy, wild-type cynomolgus monkeys (n=4 per group), plus a control group (data are normalized to aCSF). Necropsy was performed 2 weeks after last dose.

Figure 7 (left) shows CMP-002 concentration (y-axis; normalized on a 0-100 scale) across six regions of the brain: frontal cortex, temporal cortex, parietal cortex, motor cortex, occipital cortex, and hippocampus. The exact dose levels are not disclosed, but are low / mid / high. The data show a dose-dependent increase in CMP-002 concentration across all of the listed brain regions, with no region below the ~50 range, which suggests CMP-002 is reaching all six of the listed regions.

PD follows the PK effect - dose-dependent increases in SYNGAP1 protein. In Figure 7 (right), the data shows SYNGAP1 protein levels from the frontal cortex (highest CMP-002 concentration at high dose) and the hippocampus (lowest CMP-002 concentration at high dose). SYNGAP1 protein levels increase in a dose-dependent manner in both the frontal cortex and the hippocampus, and from our estimation of the graph, we see a ~30-60% increase in SYNGAP protein levels at the high dose. This tracks well with the PK data and CMP-002 levels shown in the figure on the right.

Collectively, this data supports our confidence in the PK/PD correlation and the ability of CMP-002 to get into clinically relevant regions of the brain. The magnitude of the protein increase (~30-60%) is also clinically relevant if one assumes a similar increase in haploinsufficient SYNGAP1 protein levels of ~50% of normal at baseline, that would put near the wild-type or normal levels, which also supports the mouse model data discussed above.

Figure 7 - Intrathecal administration of CMP-002 in NHPs leads to broad, dose-dependent distribution across disease-relevant brain regions and a ~1.3-1.6x increase in SYNGAP1 protein levels

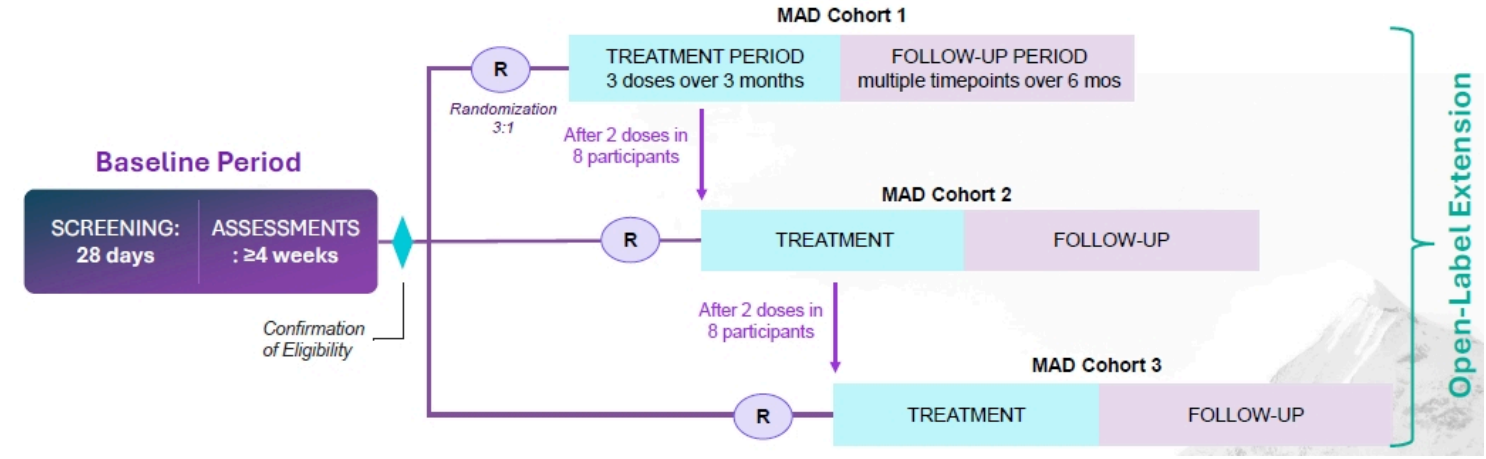


Source: CAMP presentation Sep 2026

CMP-002: PHASE 1/2 ASCEND TRIAL IN SYNGAP1

CAMP disclosed the Phase 1/2 ASCEND trial design during a virtual analyst day on 9/28/26. The trial design and descriptions are shown below.

Figure 8 - CMP-002 Phase 1/2 ASCEND Trial in SYNGAP1



Source: CAMP presentation Sep 2026

CMP-002 Phase 1/2 ASCEND Trial. Patients will be randomized 3:1 (CAMP-002 intrathecal injection : sham pinprick control) and an open-label extension (OLE) will be offered to all eligible patients. The trial will be conducted in 11+ sites across 9 countries and 5 regulatory regions (4 sites in already-approved regions; 7 EU sites pending EU review)

Population: ≥8 participants per cohort; ages 2 to <18; genetically confirmed SYNGAP1 with protein-truncating mutations in exons 5-19; required daily seizures, impaired sleep, inability to speak in phrases; refractory epilepsy (daily seizures despite ≥1 ongoing ASM and prior trial of ≥2 ASMs); stable ASM regimen at baseline

Cohort escalation: ≥4-week baseline; treatment period of 3 doses over 3 months; 6-month follow-up → ~10 months total per participant. After 2 doses in 8 participants, Safety Review Committee reviews all safety data before next-cohort dosing

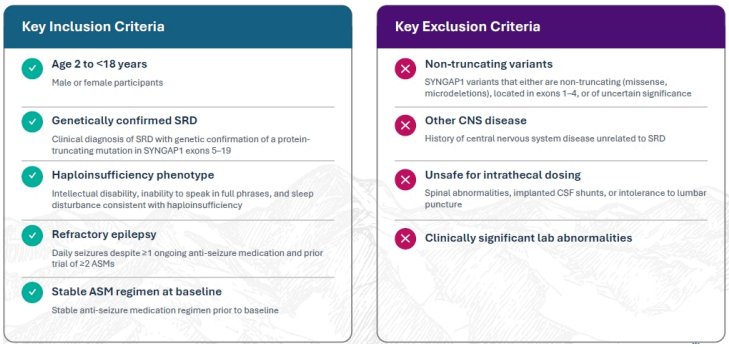
Efficacy objectives: includes seizures, sleep, motor/gait, communication, behavior, development, cognition. A summary of the scales being used to measure behavior, development, communication, and motor function are shown in Figure 11. Video EEG, seizure diaries, the SIRIUS seizure-impact scale, Children's Sleep Habits Questionnaire, and actigraphy will also be utilized

Figure 9 - CMP-002 Phase 1/2 Trial in SYNGAP1 - Overview

Design	• Phase 1/2, randomized, double-blind, controlled, multiple ascending dose study • 3:1 randomization • IT administration or a sham pinprick • Open-Label Extension
Study population	• Minimum 8 participants per cohort • Participants ≥2 to <18 years of age with genetically confirmed SYNGAP1-Related Disorder (SRD) • Required daily seizures, impaired sleep, and inability to speak in phrases
Study periods	• Screening and baseline period • Study treatment period with 3 doses • Post-treatment follow-up
Study length	• ~10 months per participant

Source: CAMP presentation Sep 2026

Figure 10 - CMP-002 Phase 1/2 Trial in SYNGAP1 - Eligibility Criteria



Source: CAMP presentation Sep 2026

Figure 11 - CMP-002 Ph1/2 ASCEND Trial in SYNGAP1 - validated scales to be used to assess behavior, development, communication, and motor function

Scale	Communication	Motor	Behavior	Development
Bayley Scales of Infant and Toddler Development Bayley-4		✓	✓	✓
Vineland Adaptive Behavior Scales, 3rd Edition Vineland-3	✓	✓	✓	✓
Aberrant Behavior Checklist, 2nd Edition ABC-2	✓		✓	
Observer-Reported Communication Ability Measure ORCA	✓			
Gross Motor Function Measure GMFM		✓		

Leveraging validated scales used in DEE clinical trials and ProMMiS Natural History Study

✓ Scale to be utilized to assess domain in ASCEND trial

CAMP4

Source: CAMP presentation Sep 2026

Our key takeaways from the CAMP analyst day on 9/28/26 as it pertains to the CMP-002 Phase 1/2 Trial in SYNGAP1:

- **Seizure reduction as a potential registrational endpoint.** CAMP mgmt noted that as a 1st-in-disease program (no approved drugs for SYNGAP1) they can't yet define a meaningful threshold for success; regulatory discussions have centered around having reliable and validated ways to assess changes across multiple methods rather than agreeing on a specific number (e.g. seizure reduction %). Mgmt has engaged regulators globally and is intentionally trying to keep them "open-minded"; CAMP mgmt declined to pre-define/ pre-commit to clinical endpoints, emphasizing reliable, validated methodologies so that any observed movement is credible.

- **Dose-response expectations.** Mgmt has not yet disclosed the dose levels but noted that all three dose levels are "within the expected therapeutic range", framing the initial low starting dose as a safety-oriented, peer-consistent approach.

- **Natural history study comparison.** Mgmt acknowledged natural history (NH) visit cadence (often ~6-monthly) is less frequent than the visits on the Phase 1/2 ASCEND trial but noted NH assesses a broader window; they highlighted the ≥4-week baseline and the internal sham control for comparison. The Phase 1/2 ASCEND trial is **not powered for statistical significance** vs. control, but numerical comparisons can be made.

- **Timing of data disclosure - 1H28 initial topline disclosure is a conservative base case.** Mgmt's goal is to deliver a high-quality trial with high confidence in the results that also provides development pathway optionality going forward. They will not be reviewing data on a streaming/routine basis to protect the double-blind integrity of the trial (important given caregiver/clinician-rated assessments). Certain (undisclosed) pre-specified review points exist but interim commentary or disclosure is not promised. Mgmt noted there could be potential scenarios in which exceptional data could warrant early/interim data disclosure, but noted these are unpredictable and reiterated its stance on delivering high quality data.

PIPELINE BEYOND SYNGAP1

- **SHANK3 (Phelan-McDermid Syndrome):** SHANK3 is a post-synaptic scaffolding protein and haploinsufficiency presents a phenotype that is very similar to SYNGAP1. The estimated US patient prevalence is ≥45,000 and existing natural history study/registry have >3,500 patients. CAMP is planning to nominate a development candidate in 2027. Preclinical data translation to the clinic relies more on primate data (plus related mouse models) as the humanized-mouse model is less established than SYNGAP1's.

- **Two additional brain haploinsufficiency programs** are expected to be disclosed over the next year, which mgmt has framed as franchise-leveraging shared brain targets, with intrathecal delivery and a haploinsufficiency mechanism.

SYNGAP1 COMPETITIVE LANDSCAPE

Praxis Precision Medicines (PRAX): PRAX-090: utilizes the Solidus ASO platform, which employs computational design for bi-directional tuning (up or down regulation of a target transcript). PRAX-090 is designed to upregulate SYNGAP1 mRNA and protein to address SYNGAP1-related loss-of-function (LoF) mutations.

Summary of past results: no results presented to date, preclinical proof-of-mechanism only

Current status: Preclinical. PRAX is guiding to nominate a development candidate for each of its three early-stage ASO programs, including PRAX-090 for SYNGAP1, although timelines for the SYNGAP1 program are not clear at this time

Stoke Therapeutics (STOK) / Acadia Pharmaceuticals (ACAD): utilizes the TANGO (Targeted Augmentation of Nuclear Gene Output) platform to design ASOs that reduce non-productive splicing and upregulate productive mRNA/protein. Announced in January 2022, the collaboration includes SYNGAP1, Rett Syndrome (MECP2), and one undisclosed CNS target; for SYNGAP1 the companies share WW R&D and commercialization 50/50 on WW costs and profits, with Stoke eligible for development, regulatory, and sales-based milestones. Acadia made an upfront payment of $60M. Stoke leads research/preclinical work on Rett and undisclosed targets, but SYNGAP1 is the 50/50 co-development and co-commercialization partnership.

Summary of past results: no clinical results to date

Current status: discovery/lead optimization. SYNGAP1 clinical candidate nomination could come in 2026

Quiver Biosciences (private): Utilizes its proprietary Genomic Positioning System (GPS) platform, which is a scalable single-cell human neuronal electrophysiology platform paired with AI/ML analytics and ASO design. The SYNGAP1 program is an ASO for the DEE presentation.

Summary of past results: none disclosed

Current status: listed in the company's pipeline as SYNGAP1 in the early discovery phase

GondolaBio (private): BridgeBio sister company, launched in 2024 with $300M.

Summary of past results: none disclosed

Current status: SYNGAP1 appears in the GondolaBio pipeline in the discovery/lead op stage

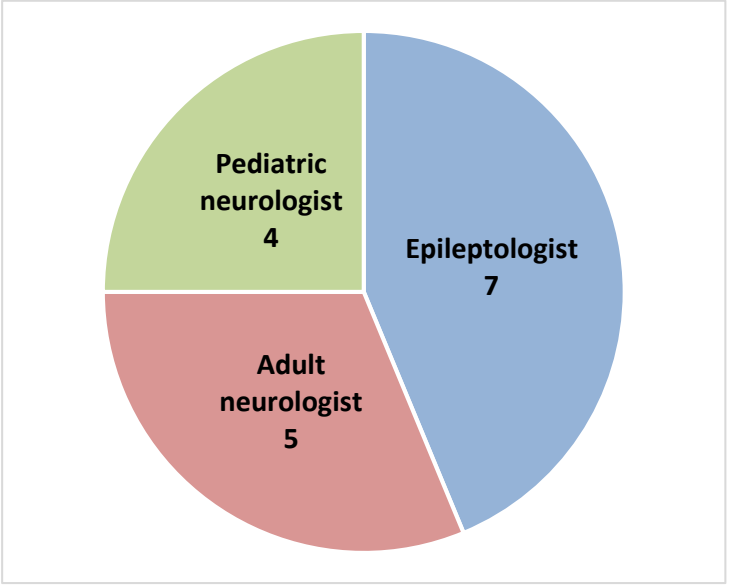
PHYSICIAN SURVEY ON SYNGAP1

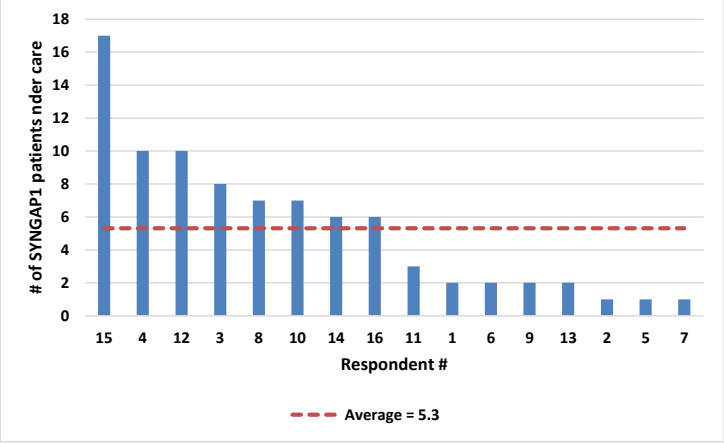
We conducted a survey of US physicians (n=16; epileptologists & neurologists) who treat SYNGAP1 patients - the survey was conducted 9/23/26 through 9/28/26 using Guidepoint.

We believe the results of this survey highlight the significant unmet need and commercial opportunity in SYNGAP1, and bode well for CAMP's development of CMP-002.

Question 1: What is your primary specialty? How many patients with genetically confirmed SYNGAP1-related disorder do you currently actively manage?

Respondent	State	Specialty	# of SYNGAP1 patients	number of patients you suspect have SYNGAP1 but who are genetically unconfirmed
1	NY	Epileptologist	2	0
2	NY	Adult neurologist	1	0
3	NY	Epileptologist	8	1
4	IL	Pediatric neurologist	10	0
5	NY	Adult neurologist	1	0
6	NV	Adult neurologist	2	0
7	FL	Pediatric neurologist	1	0
8	WA	Epileptologist	7	3
9	TN	Adult neurologist	2	0
10	PA	Epileptologist	7	2
11	OR	Epileptologist	3	0
12	WA	Adult neurologist	10	3
13	NC	Pediatric neurologist	2	0
14	AZ	Epileptologist	6	10
15	AR	Pediatric neurologist	17	7
16	NJ	Epileptologist	6	0
		Average	*5.3*	
		Median	*4.5*	

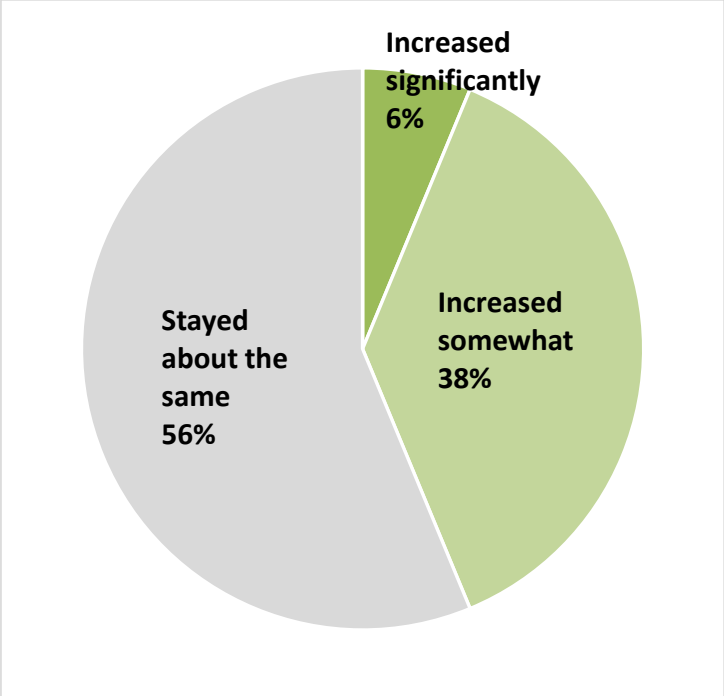




- **Small, concentrated patient base with a sizable undiagnosed pool.** The panel manages about 85 SYNGAP1 patients (mean 5.3, median 4.5), concentrated in a few high-volume physicians (one manages 17). Respondents report 71 genetically confirmed patients and 26 suspected but unconfirmed. Suspected cases are concentrated in three respondents (10, 7 and 3), and 10 of 16 report none.

Question 2: Over the past 3 years, the number of SYNGAP1 patients you diagnose or manage has:

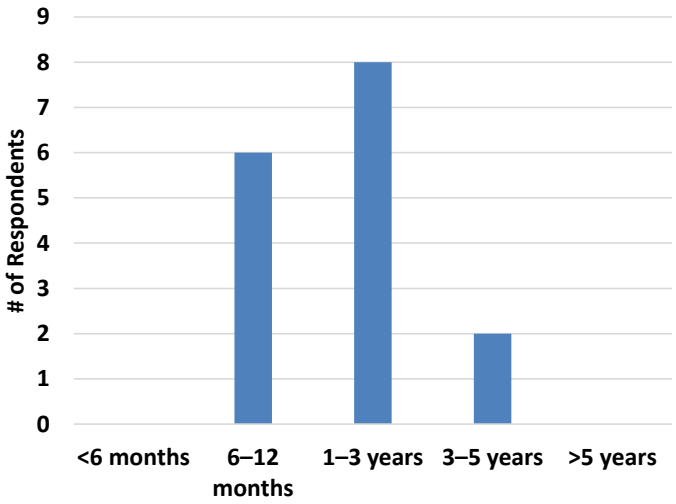
Respondent	State	Specialty	# of SYNGAP1 patients	Increased significantly	Increased somewhat	Stayed about the same	Decreased	Explanation
1	NY	Epileptologist	2			X		it hasn't changed. Its a rare condition that we don't see every year.
2	NY	Adult neurologist	1			X		Only have 1 patient
3	NY	Epileptologist	8			X		no meaningful inc or dec
4	IL	Pediatric neurologist	10		X			More frequent usage of genetic testing
5	NY	Adult neurologist	1			X		Disease itself is very rare and I have only seen two cases, one of which I am currently managing.
6	NV	Adult neurologist	2			X		same
7	FL	Pediatric neurologist	1			X		I have diagnosed about one patient every 1-2 years
8	WA	Epileptologist	7			X		No change
9	TN	Adult neurologist	2		X			better recognition and more reliable/available testing have improved diagonsis
10	PA	Epileptologist	7		X			Genetic phenotyping is now more accessible
11	OR	Epileptologist	3		X			Went from 1 to 3
12	WA	Adult neurologist	10		X			Increased awareness and screening with genetic panels
13	NC	Pediatric neurologist	2			X		stayed about the same
14	AZ	Epileptologist	6		X			I have gotten more insurance coverage for epilepsy gene testing, and thus the incidence of finding pathogenic variants to SYNGAP1 is increasing.
15	AR	Pediatric neurologist	17	X				more testing
16	NJ	Epileptologist	6			X		very rare disease
			Count	1	6	9	0	
			%	*6%*	*38%*	*56%*	*0%*	

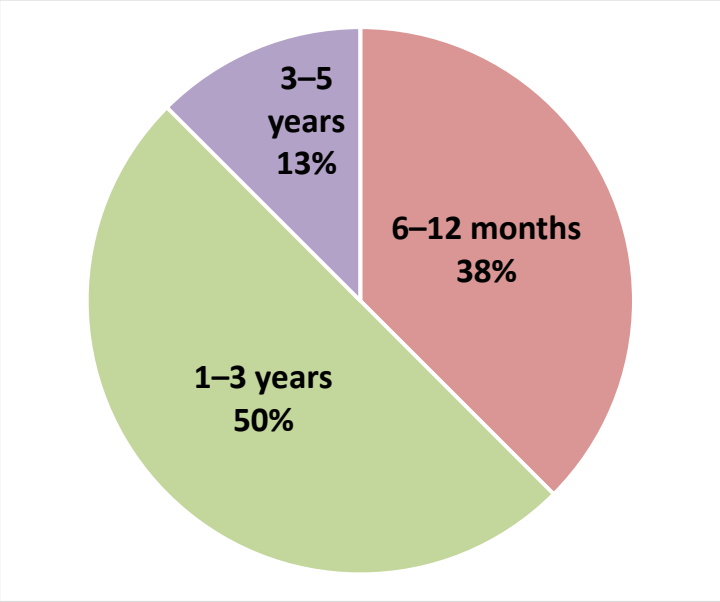


- **Patient volumes are stable to growing, with growth driven by testing, not prevalence.** No physician reported a decline in diagnosis. 56% say patient numbers are flat, 38% say they rose somewhat and 6% say significantly. Respondents who saw growth credit more genetic testing, better insurance coverage for epilepsy panels and greater awareness.

Question 3: In your experience, what is the typical time from first symptoms to a confirmed SYNGAP1 diagnosis?

Respondent	State	Specialty	# of SYNGAP1 patients	<6 months	6–12 months	1–3 years	3–5 years	>5 years
1	NY	Epileptologist	2			X		
2	NY	Adult neurologist	1			X		
3	NY	Epileptologist	8			X		
4	IL	Pediatric neurologist	10		X			
5	NY	Adult neurologist	1			X		
6	NV	Adult neurologist	2			X		
7	FL	Pediatric neurologist	1		X			
8	WA	Epileptologist	7				X	
9	TN	Adult neurologist	2		X			
10	PA	Epileptologist	7		X			
11	OR	Epileptologist	3			X		
12	WA	Adult neurologist	10		X			
13	NC	Pediatric neurologist	2		X			
14	AZ	Epileptologist	6				X	
15	AR	Pediatric neurologist	17			X		
16	NJ	Epileptologist	6			X		
		Count		0	6	8	2	0
		%		0%	38%	50%	13%	0%

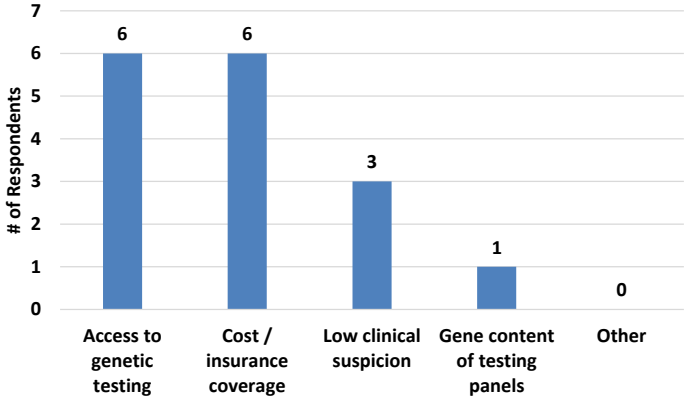


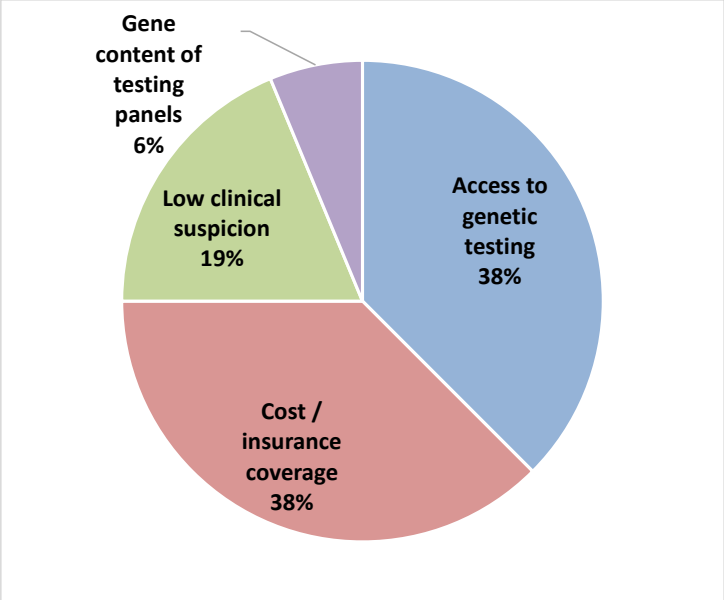


- **Diagnosis typically takes a year or more after symptom onset.** Half say 1-3 years, 38% say 6-12 months and 13% say 3-5 years. No one reports under 6 months or over 5 years.

Question 4: What is the single biggest bottleneck to a confirmed SYNGAP1 diagnosis?

Respondent	State	Specialty	# of SYNGAP1 patients	Access to genetic testing	Cost / insurance coverage	Low clinical suspicion	Gene content of testing panels	Other
1	NY	Epileptologist	2	X				
2	NY	Adult neurologist	1		X			
3	NY	Epileptologist	8	X				
4	IL	Pediatric neurologist	10			X		
5	NY	Adult neurologist	1			X		
6	NV	Adult neurologist	2		X			
7	FL	Pediatric neurologist	1	X				
8	WA	Epileptologist	7		X			
9	TN	Adult neurologist	2		X			
10	PA	Epileptologist	7		X			
11	OR	Epileptologist	3	X				
12	WA	Adult neurologist	10	X				
13	NC	Pediatric neurologist	2				X	
14	AZ	Epileptologist	6		X			
15	AR	Pediatric neurologist	17	X				
16	NJ	Epileptologist	6			X		
		Count		6	6	3	1	0
		%		38%	38%	19%	6%	0%

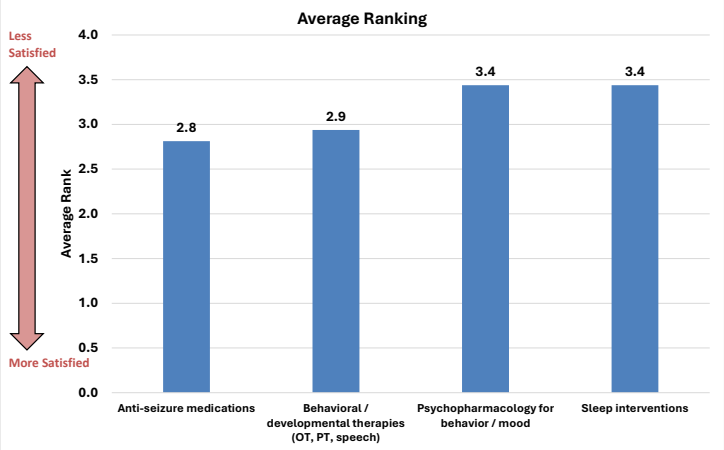


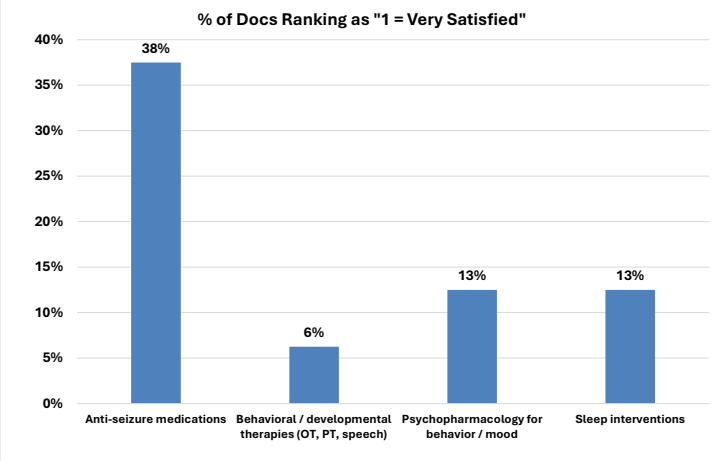


- **Testing access and cost, not clinical suspicion, are the main diagnostic barriers.** Access to genetic testing and cost/ insurance coverage tie at ~38% each, so 75% point to testing-related barriers. Low clinical suspicion is 19% and panel gene content is 6% (see the data note below).

Question 5: How do you currently manage your SYNGAP1 patients, and how satisfied are you with each approach? Rate each row: 1 = very satisfied 2 = satisfied 3 = neutral 4 = dissatisfied 5 = very dissatisfied N/A = do not use

Respondent	State	Specialty	# of SYNGAP1 patients	Anti-seizure medications	Behavioral / developmental therapies (OT, PT, speech)	Psychopharmacology for behavior / mood	Sleep interventions
1	NY	Epileptologist	2	2	3	3	3
2	NY	Adult neurologist	1	2	3	4	4
3	NY	Epileptologist	8	3	3	2	2
4	IL	Pediatric neurologist	10	3	2	2	4
5	NY	Adult neurologist	1	2	2	4	3
6	NV	Adult neurologist	2	3	3	3	3
7	FL	Pediatric neurologist	1	2	3	4	4
8	WA	Epileptologist	7	2	4	5	4
9	TN	Adult neurologist	2	3	4	4	4
10	PA	Epileptologist	7	4	2	4	4
11	OR	Epileptologist	3	3	4	4	4
12	WA	Adult neurologist	10	4	3	3	3
13	NC	Pediatric neurologist	2	4	4	3	3
14	AZ	Epileptologist	6	2	2	4	4
15	AR	Pediatric neurologist	17	3	4	3	4
16	NJ	Epileptologist	6	3	1	3	2
		Average		**2.8**	**2.9**	**3.4**	**3.4**
		Median		**3.0**	**3.0**	**3.5**	**4.0**
		High (very dissatisfied)		**4**	**4**	**5**	**4**
		Low (very satisfied)		**2**	**1**	**2**	**2**
		Count - High (very dissatisfied)		**19%**	**31%**	**6%**	**56%**
		Count - Low (very satisfied)		**38%**	**6%**	**13%**	**13%**

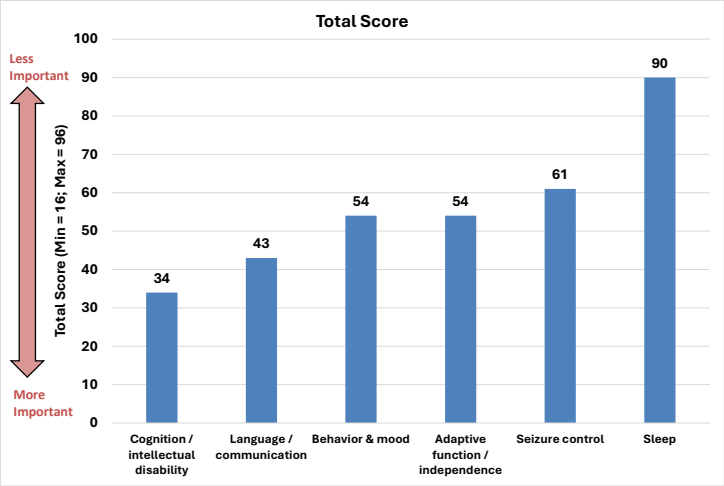


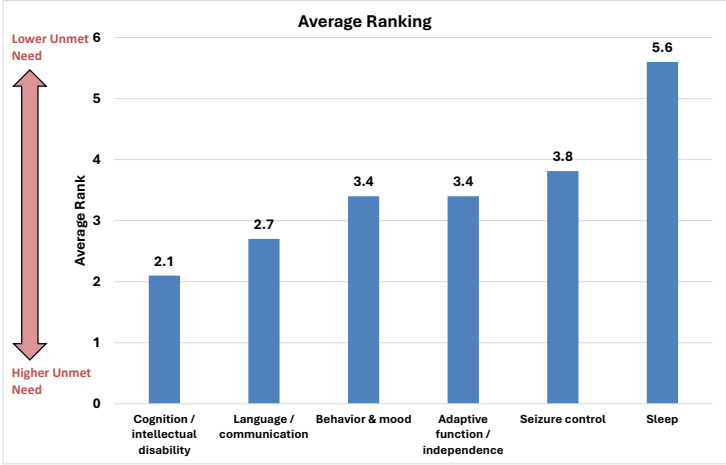


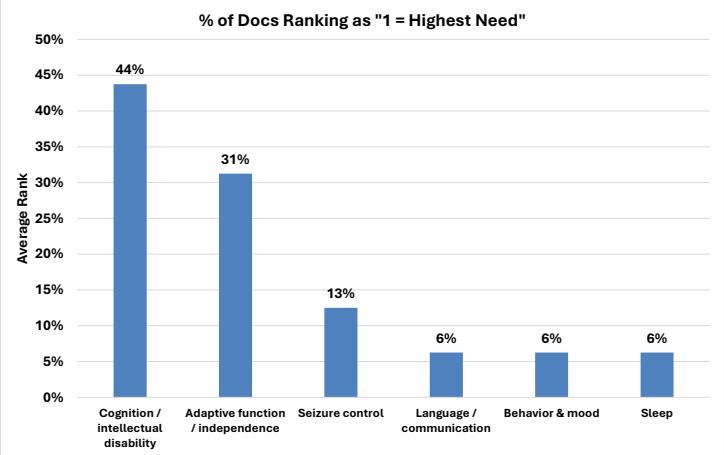
- **Current SYNGAP1 management leaves room for improvement, especially for sleep and behavior/mood.** No approach clearly satisfies physicians. Anti-seizure medications score best (avg 2.8/5.0) and behavioral/developmental therapies are polarized (avg 2.9/5.0). Psychopharmacology (avg 3.4/5.0) and sleep interventions (avg 3.4/5.0) are weakest, with 50% and 56% of respondents dissatisfied, respectively.

Question 6: Across symptom domains, where is the greatest unmet therapeutic need for your SYNGAP1 patients? Select your top 3 and rank them: 1 = highest need, 2 = second, 3 = third, … 6 = least unmet need.

Respondent	State	Specialty	# of SYNGAP1 patients	Seizure control	Cognition / intellectual disability	Language / communication	Behavior & mood	Adaptive function / independence	Sleep
1	NY	Epileptologist	2	2	1	3	4	5	6
2	NY	Adult neurologist	1	6	4	3	2	1	5
3	NY	Epileptologist	8	4	2	3	5	1	6
4	IL	Pediatric neurologist	10	3	1	2	4	5	6
5	NY	Adult neurologist	1	5	1	2	3	6	4
6	NV	Adult neurologist	2	1	2	4	3	5	6
7	FL	Pediatric neurologist	1	5	4	3	2	1	6
8	WA	Epileptologist	7	6	3	4	1	2	5
9	TN	Adult neurologist	2	5	1	2	4	3	6
10	PA	Epileptologist	7	1	2	3	4	5	6
11	OR	Epileptologist	3	6	3	1	2	4	5
12	WA	Adult neurologist	10	2	1	3	4	5	6
13	NC	Pediatric neurologist	2	5	3	2	4	1	6
14	AZ	Epileptologist	6	2	1	3	4	5	6
15	AR	Pediatric neurologist	17	6	1	2	3	4	5
16	NJ	Epileptologist	6	2	4	3	5	1	6
			Average	**3.8**	**2.1**	**2.7**	**3.4**	**3.4**	**5.6**
			Median	**4.5**	**2.0**	**3.0**	**4.0**	**4.0**	**6.0**
			High (lower need)	**6**	**4**	**4**	**5**	**6**	**6**
			Low (higher need)	**1**	**1**	**1**	**1**	**1**	**4**
			Count - High (lower need)	**25%**	**19%**	**13%**	**13%**	**6%**	**69%**
			Count - Low (higher need)	**13%**	**44%**	**6%**	**6%**	**31%**	**6%**



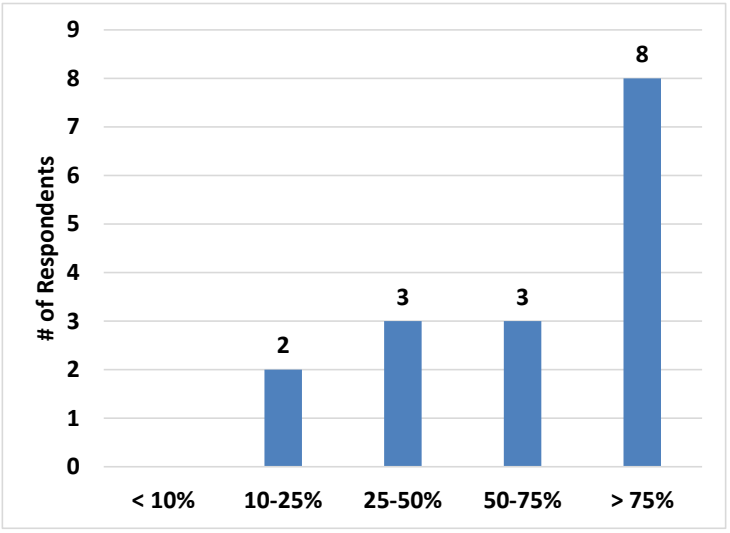


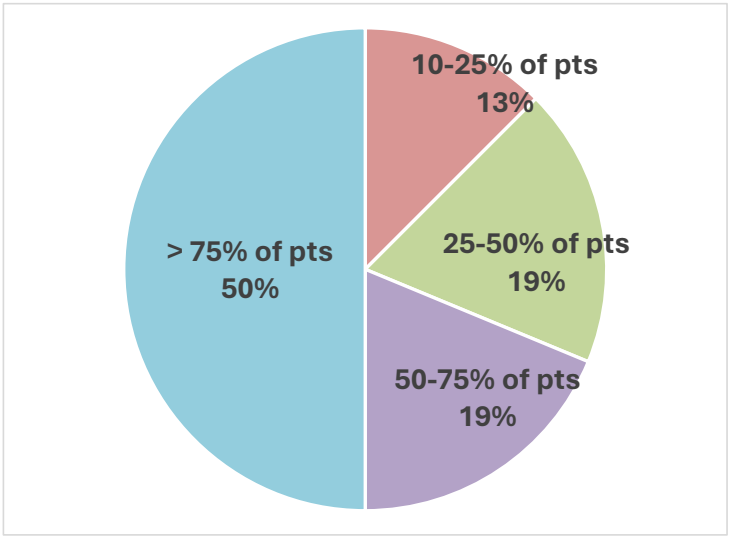


- **Cognition is the top unmet need, followed by communication and adaptive function; seizures and sleep rank lower.** Cognition/intellectual disability ranks first (avg rank 2.1/6.0, #1 for 44%) and language/communication is second (2.7/6.0). Adaptive function is polarized (31% rank it #1, but avg 3.4/6.0). Seizure control ranks lower in unmet need than expected (avg 3.8/6.0), consistent with relative satisfaction with anti-seizure medications. Sleep is clearly last (avg 5.6/6.0).

Question 7: Consider a disease-modifying therapy that targets the underlying SYNGAP1 haploinsufficiency — administered by intrathecal injection, designed to raise SYNGAP protein toward normal levels and improve outcomes across multiple domains. Assume a favorable safety and efficacy profile. Of the SYNGAP1 patients you manage, approximately what proportion would you consider clinically appropriate candidates?

Respondent	State	Specialty	# of SYNGAP1 patients	< 10%	10-25%	25-50%	50-75%	> 75%
1	NY	Epileptologist	2			X		
2	NY	Adult neurologist	1					X
3	NY	Epileptologist	8				X	
4	IL	Pediatric neurologist	10					X
5	NY	Adult neurologist	1					X
6	NV	Adult neurologist	2		X			
7	FL	Pediatric neurologist	1					X
8	WA	Epileptologist	7			X		
9	TN	Adult neurologist	2				X	
10	PA	Epileptologist	7					X
11	OR	Epileptologist	3					X
12	WA	Adult neurologist	10				X	
13	NC	Pediatric neurologist	2		X			
14	AZ	Epileptologist	6					X
15	AR	Pediatric neurologist	17			X		
16	NJ	Epileptologist	6					X
			Count	0	2	3	3	8
			%	0%	13%	19%	19%	50%

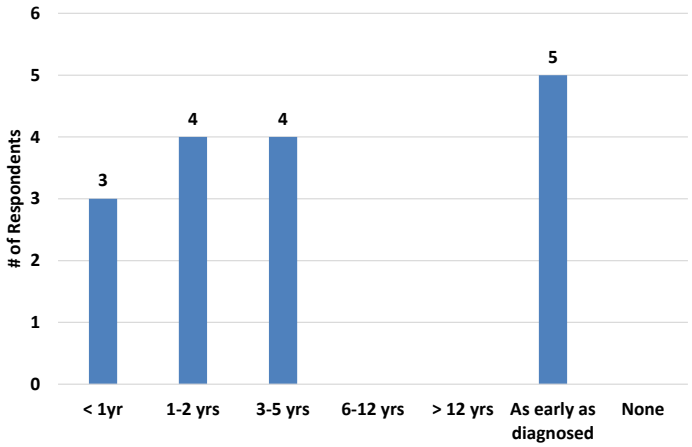


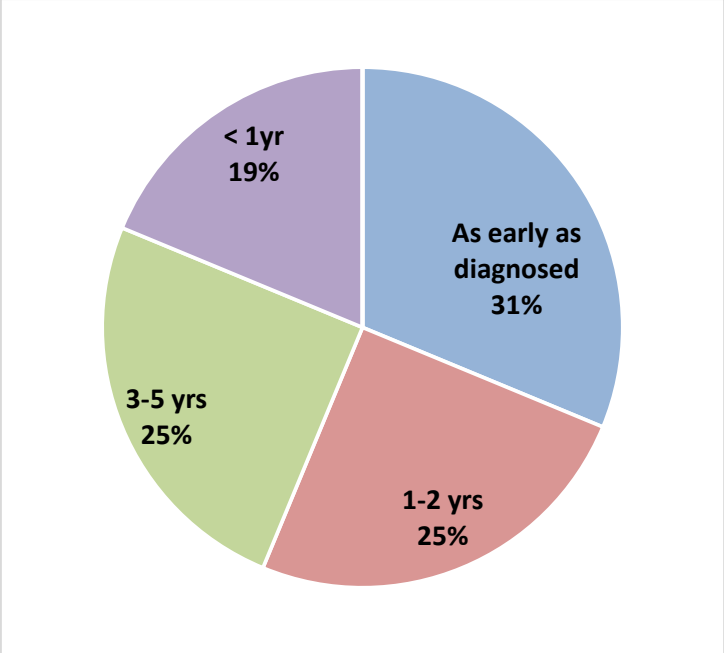


- **Physicians see a broad addressable population for a disease-modifying therapy.** Half would consider more than 75% of their patients appropriate candidates, and 69% would consider more than half. No one chose under 10%.

Question 8: Consider a disease-modifying therapy that targets the underlying SYNGAP1 haploinsufficiency — administered by intrathecal injection, designed to raise SYNGAP protein toward normal levels and improve outcomes across multiple domains. Assume a favorable safety and efficacy profile. What is the youngest age at which you would consider initiating a disease-modifying therapy?

Respondent	State	Specialty	# of SYNGAP1 patients	< 1yr	1-2 yrs	3-5 yrs	6-12 yrs	> 12 yrs	As early as diagnosed	None
1	NY	Epileptologist	2		x					
2	NY	Adult neurologist	1	x						
3	NY	Epileptologist	8						x	
4	IL	Pediatric neurologist	10	x						
5	NY	Adult neurologist	1			x				
6	NV	Adult neurologist	2		x					
7	FL	Pediatric neurologist	1			x				
8	WA	Epileptologist	7						x	
9	TN	Adult neurologist	2		x					
10	PA	Epileptologist	7			x				
11	OR	Epileptologist	3		x					
12	WA	Adult neurologist	10						x	
13	NC	Pediatric neurologist	2			x				
14	AZ	Epileptologist	6						x	
15	AR	Pediatric neurologist	17	x						
16	NJ	Epileptologist	6						x	
			Count	3	4	4	0	0	5	0
			%	19%	25%	25%	0%	0%	31%	0%

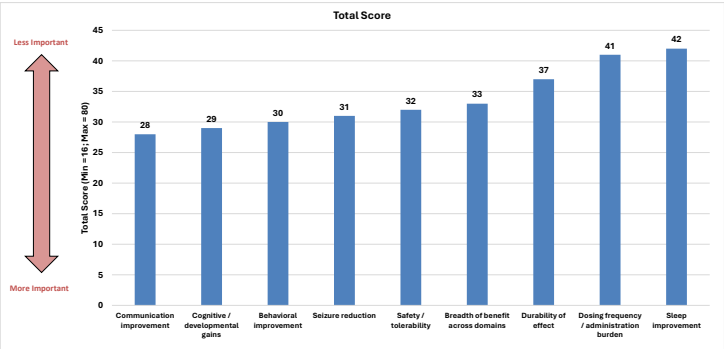




- **Most physicians would treat early, with 75% willing to start by age 2.** 31% would start as early as diagnosed, 19% under 1 year and 25% at 1-2 years. The remaining 25% would start at 3-5 years, and no one would wait past 5.

Question 9: How important would each of the following be in your decision to adopt such a therapy? Rate each row: 1 = extremely important … 5 = not at all important

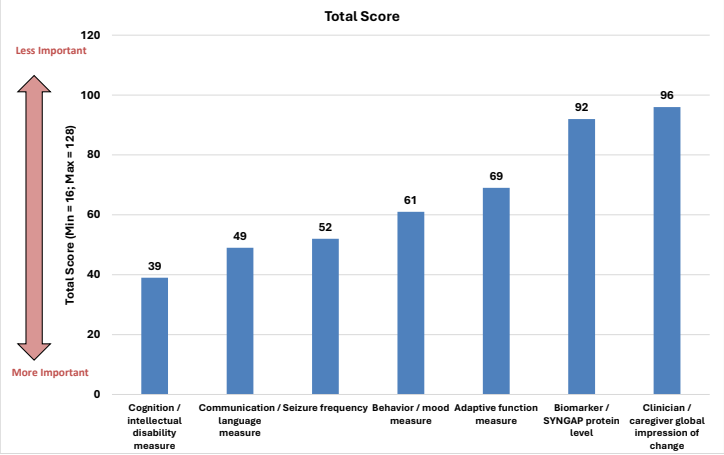
Respondent	State	Specialty	# of SYNGAP1 patients	Attribute / outcome	Cognitive / developmental gains	Seizure reduction	Behavioral improvement	Communication improvement	Sleep improvement	Safety / tolerability	Durability of effect	Breadth of benefit across domains	Dosing frequency / administration burden
1	NY	Epileptologist	2	2	1	1	2	1	3	1	2	2	3
2	NY	Adult neurologist	1	1	1	1	1	1	1	1	1	1	2
3	NY	Epileptologist	8	5	5	5	5	5	4	4	5	4	3
4	IL	Pediatric neurologist	10	2	1	1	3	2	4	2	3	3	4
5	NY	Adult neurologist	1	1	2	1	1	1	3	2	3	1	2
6	NV	Adult neurologist	2	4	4	5	4	4	4	4	5	4	4
7	FL	Pediatric neurologist	1	1	2	3	2	2	2	1	2	1	2
8	WA	Epileptologist	7	3	1	2	1	1	2	3	2	2	1
9	TN	Adult neurologist	2	2	1	2	1	2	2	1	2	1	2
10	PA	Epileptologist	7	2	1	1	1	1	2	1	1	1	2
11	OR	Epileptologist	3	1	2	1	1	1	3	2	2	3	2
12	WA	Adult neurologist	10	3	2	1	2	1	3	2	2	3	3
13	NC	Pediatric neurologist	2	1	1	1	1	1	1	1	1	1	1
14	AZ	Epileptologist	6	1	1	1	1	1	2	1	2	2	3
15	AR	Pediatric neurologist	17	4	3	4	3	3	3	4	3	3	4
16	NJ	Epileptologist	6	1	1	1	1	1	3	2	1	1	3
		Average		**2.1**	**1.8**	**1.9**	**1.9**	**1.8**	**2.6**	**2.0**	**2.3**	**2.1**	**2.6**
		Median		**2**	**1**	**1**	**1**	**1**	**3**	**2**	**2**	**2**	**3**
		High (not at all important)		**5**	**5**	**5**	**5**	**5**	**4**	**4**	**5**	**4**	**4**
		Low (extremely important)		**1**	**1**	**1**	**1**	**1**	**1**	**1**	**1**	**1**	**1**
		% w/ high value (not at all important)		**6%**	**6%**	**13%**	**6%**	**6%**	**19%**	**19%**	**13%**	**13%**	**19%**
		% w/ low value (extremely important)		**44%**	**56%**	**63%**	**56%**	**63%**	**13%**	**44%**	**25%**	**44%**	**13%**

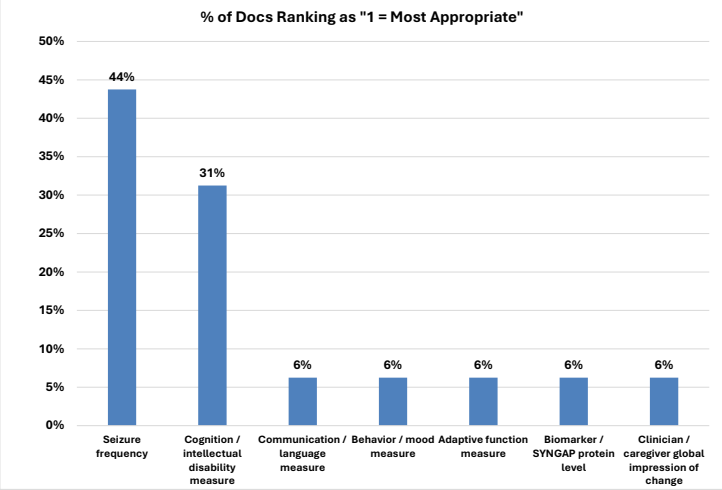


- **Efficacy drives adoption, while the intrathecal administration burden is a minor concern.** Communication improvement (avg 1.75/5.0), cognitive gains (1.81/5.0), behavioral improvement (1.88/5.0) and seizure reduction (1.94/5.0) are rated "extremely important" by 56-63%. Safety (2.0/5.0) and breadth of benefit (2.1/5.0) follow, then durability (2.3/5.0). Sleep improvement and dosing/administration burden are least important (avg ~2.6/5.0 each).

Question 10: Which clinical endpoint would you consider most appropriate to demonstrate benefit in a SYNGAP1 trial? Please rank the following with: 1 = most appropriate.

Respondent	State	Specialty	# of SYNGAP1 patients	Seizure frequency	Cognition / intellectual disability measure	Communication / language measure	Behavior / mood measure	Adaptive function measure	Clinician / caregiver global impression of change	Biomarker / SYNGAP protein level
1	NY	Epileptologist	2	1	2	3	4	5	6	7
2	NY	Adult neurologist	1	7	3	4	5	2	6	1
3	NY	Epileptologist	8	1	3	2	4	5	6	7
4	IL	Pediatric neurologist	10	3	2	1	4	5	6	7
5	NY	Adult neurologist	1	1	5	4	7	2	6	3
6	NV	Adult neurologist	2	1	3	4	5	7	8	2
7	FL	Pediatric neurologist	1	5	3	4	2	1	6	7
8	WA	Epileptologist	7	7	1	2	3	4	5	6
9	TN	Adult neurologist	2	6	1	2	3	4	8	5
10	PA	Epileptologist	7	1	2	3	4	5	6	7
11	OR	Epileptologist	3	2	1	3	4	6	5	7
12	WA	Adult neurologist	10	4	1	2	3	8	7	6
13	NC	Pediatric neurologist	2	1	2	8	3	4	5	6
14	AZ	Epileptologist	6	3	1	2	5	4	6	7
15	AR	Pediatric neurologist	17	8	6	3	1	2	4	7
16	NJ	Epileptologist	6	1	3	2	4	5	6	7
		Average		**3.3**	**2.4**	**3.1**	**3.8**	**4.3**	**6.0**	**5.8**
		Median		**3**	**2**	**3**	**4**	**5**	**6**	**7**
		Total		**52**	**39**	**49**	**61**	**69**	**96**	**92**
		High (least appropriate)		**8**	**6**	**8**	**7**	**8**	**8**	**7**
		Low (most appropriate)		**1**	**1**	**1**	**1**	**1**	**4**	**1**
		Count - High (least appropriate)		**6%**	**6%**	**6%**	**6%**	**6%**	**13%**	**56%**
		Count - Low (most appropriate)		**44%**	**31%**	**6%**	**6%**	**6%**	**6%**	**6%**

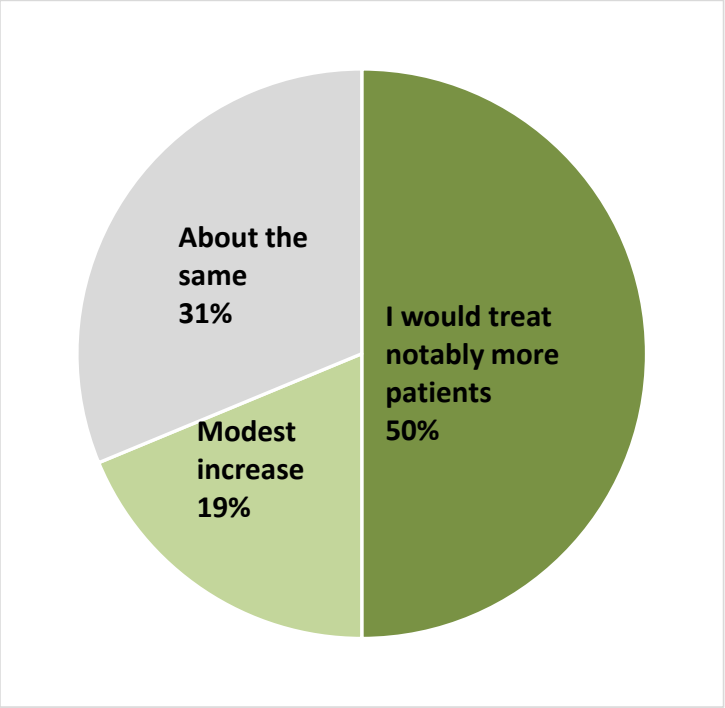




- **Seizure frequency and cognition are the preferred trial endpoints; the protein biomarker and global impression are not.** One of the two is top choice for 75% (seizure frequency #1 for 44%, cognition #1 for 31%). Cognition has the best average rank (2.4/7.0, vs 3.3/7.0 for seizures), followed by communication (3.1/7.0). SYNGAP1 protein biomarker (avg 5.75/7.0) and clinician/caregiver global impression (avg 6.0/7.0) rank lowest.

Question 11: If a safe and effective disease-modifying therapy were approved, how would your treated SYNGAP1 population change?

Respondent	State	Specialty	# of SYNGAP1 patients	I would treat notably more patients	Modest increase	About the same
1	NY	Epileptologist	2			X
2	NY	Adult neurologist	1	X		
3	NY	Epileptologist	8			X
4	IL	Pediatric neurologist	10	X		
5	NY	Adult neurologist	1	X		
6	NV	Adult neurologist	2			X
7	FL	Pediatric neurologist	1		X	
8	WA	Epileptologist	7		X	
9	TN	Adult neurologist	2		X	
10	PA	Epileptologist	7	X		
11	OR	Epileptologist	3			X
12	WA	Adult neurologist	10	X		
13	NC	Pediatric neurologist	2	X		
14	AZ	Epileptologist	6	X		
15	AR	Pediatric neurologist	17	X		
16	NJ	Epileptologist	6			X
			Count	8	3	5
			%	*50%*	*19%*	*31%*



- **An approved therapy would expand the treated population for roughly 70% of physicians.** No one expects fewer patients. 50% would treat notably more, 19% a modest increase and 31% about the same.

Question 12: Do you have any additional thoughts or opinions on SYNGAP1, the unmet need, or the broader treatment landscape?

Respondent	State	Specialty	# of SYNGAP1 patients	Do you have any additional thoughts or opinions on SYNGAP1, the unmet need, or the broader treatment landscape?
1	NY	Epileptologist	2	None, I would be interested in a disease modifying therapy for any of these genetic epilepsy conditinos.
2	NY	Adult neurologist	1	Insurance is the barrier then resources and support staff on getting through the hurdles
3	NY	Epileptologist	8	would still need earlier referrals and testing
4	IL	Pediatric neurologist	10	Current unmet need is extremely high. No targeted treatment available
5	NY	Adult neurologist	1	There is significant unmet need as well as low disease visibility generally
6	NV	Adult neurologist	2	none
7	FL	Pediatric neurologist	1	N/A
8	WA	Epileptologist	7	Exciting and unique MOA
9	TN	Adult neurologist	2	rare condition, so not sure how much research is feasible
10	PA	Epileptologist	7	No
11	OR	Epileptologist	3	No
12	WA	Adult neurologist	10	Not at this time
13	NC	Pediatric neurologist	2	none
14	AZ	Epileptologist	6	We need to have better coverage of genetic testing to see more patients with this condition, and treat them with emerging disease-modifying therapies.
15	AR	Pediatric neurologist	17	No
16	NJ	Epileptologist	6	really if I specific treatment were for SYNGAP1 you will have more diagnosis given don't want to now miss these patients and miss out on a potential therapy

- **Open comments point to high unmet need and a testing/insurance bottleneck, with enthusiasm for the mechanism.** Nine of 16 gave substantive comments and seven replied "none", "no" or "N/A". Respondents say an approved therapy would drive more diagnoses. One respondent doubts how much research is feasible in such a rare disease.

REVENUE MODELS

We model unadjusted WW peak revenue of $2.1B for CMP-002 in SYNGAP1

Addressable patients: We model a prevalence of ~11,000 SYNGAP1 patients in the US ≤18 y/o, of which ~15% are currently diagnosed and are the target population for CMP-002. We model 0.5% population growth and assume the diagnosis rate increases to ~50% at peak CMP-002 market penetration.

Price: We model an annual WAC price of $575,000 at launch in 2032, which we believe is justifiable for a chronically-dosed disease-modifying therapy in an ultra-rare pediatric disorder like SYNGAP1. We considered multiple drug comps (neuromuscular, disease-modifying, genetic rare disease) and their corresponding annual WAC prices including: Amondys (~$750K/yr); Daybue (~$715K/yr); Spinraza (~$495K/yr); Onpattro (~450K/yr); Exondys (~$417K/yr); and Qalsody (~$200K/yr).

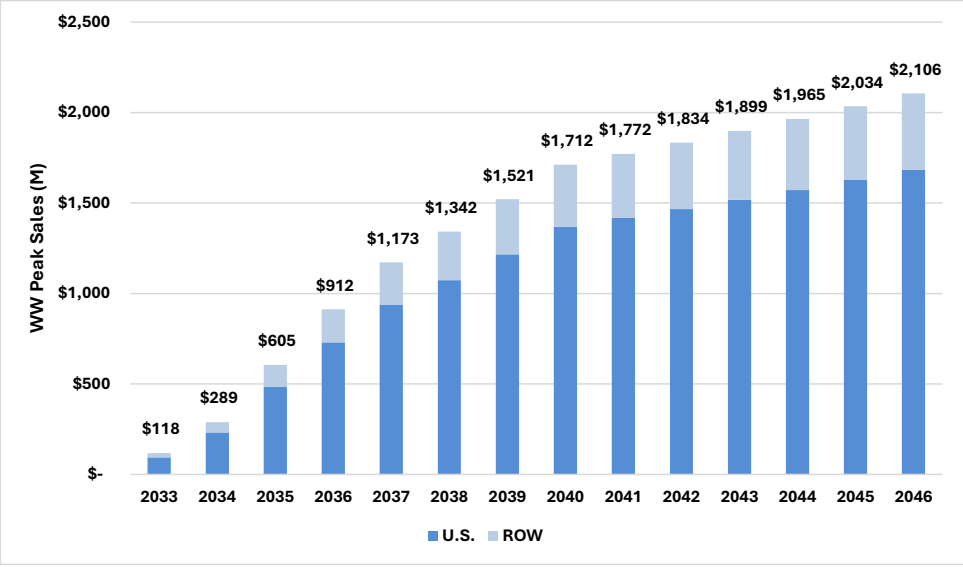
WW sales: Our WW peak sales estimate for CMP-002 in SYNGAP1 is $2.1B (~$1.7B US and ~$420M EU4 + UK). We model 50% peak penetration into the SYNGAP1 patient population, which equates to ~3,000 US patients treated at peak in 2046.

CMP-002 WW Revenue Model in SYNGAP1 (Needham & Co.)

CMP-002 WW Revenue Model In SYNGAP1 Disorders																			Patent expiry			LOE w/ HWE							
United States	**2026**	**2027**	**2028**	**2029**	**2030**	**2031**	**2032**	**2033**	**2034**	**2035**	**2036**	**2037**	**2038**	**2039**	**2040**	**2041**	**2042**	**2043**	**2044**	**2045**	**2046**	**2047**	**2048**	**2049**	**2050**	**2051**	**2052**	**2053**
SYNGAP1																												
SYNGAP1 prevalence ≤18 y/o	11,000	11,055	11,110	11,166	11,222	11,278	11,334	11,391	11,448	11,505	11,563	11,620	11,678	11,737	11,796	11,855	11,914	11,973	12,033	12,093	12,154	12,215	12,276	12,337	12,399	12,461	12,523	12,586
% diagnosed	*15%*	*16%*	*16%*	*17%*	*17%*	*18%*	*18%*	*22%*	*26%*	*30%*	*34%*	*38%*	*42%*	*46%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*
# diagnosed patients ≤18 y/o	1,650	1,714	1,778	1,842	1,908	1,974	2,040	2,506	2,976	3,452	3,931	4,416	4,905	5,399	5,898	5,927	5,957	5,987	6,017	6,047	6,077	6,107	6,138	6,169	6,199	6,230	6,262	6,293
% penetration CMP-002							*5%*	*10%*	*20%*	*35%*	*45%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*50%*	*25%*	*13%*	*6%*	*3%*	*2%*	*1%*	*0%*
Patients treated w/ CMP-002	-	-	-	-	-	-	102	251	595	1,208	1,769	2,208	2,452	2,699	2,949	2,964	2,978	2,993	3,008	3,023	3,038	1,527	767	386	194	97	49	25
WAC annual price/patient							$ 575,000	$ 592,250	$ 610,018	$ 628,318	$ 647,168	$ 666,583	$ 686,580	$ 707,177	$ 728,393	$ 750,245	$ 772,752	$ 795,934	$ 819,813	$ 844,407	$ 869,739	$ 895,831	$ 922,706	$ 950,387	$ 978,899	######	######	######
combined adherence & persistence rate							*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*	*75%*
gross:net discount							*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*	*15%*
Net annual price/patient							$ 366,563	$ 377,559	$ 388,886	$ 400,553	$ 412,569	$ 424,946	$ 437,695	$ 450,826	$ 464,350	$ 478,281	$ 492,629	$ 507,408	$ 522,630	$ 538,309	$ 554,459	$ 571,092	$ 588,225	$ 605,872	$ 624,048	$ 642,770	$ 662,053	$ 681,914
U.S. sales (M)	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ 37**	**$ 95**	**$ 231**	**$ 484**	**$ 730**	**$ 938**	**$ 1,073**	**$ 1,217**	**$ 1,369**	**$ 1,417**	**$ 1,467**	**$ 1,519**	**$ 1,572**	**$ 1,627**	**$ 1,685**	**$ 872**	**$ 451**	**$ 234**	**$ 121**	**$ 63**	**$ 32**	**$ 17**
% y/y								*153%*	*145%*	*109%*	*51%*	*29%*	*14%*	*13%*	*13%*	*4%*	*4%*	*4%*	*4%*	*4%*	*4%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*
ROW (EU4 + UK)																												
ROW sales multiplier								*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*	*0.25x*
ROW sales (M)	$ -	$ -	$ -	$ -	$ -	$ -	$ -	$ 24	$ 58	$ 121	$ 182	$ 235	$ 268	$ 304	$ 342	$ 354	$ 367	$ 380	$ 393	$ 407	$ 421	$ 218	$ 113	$ 58	$ 30	$ 16	$ 8	$ 4
ROW sales (M)	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ 24**	**$ 58**	**$ 121**	**$ 182**	**$ 235**	**$ 268**	**$ 304**	**$ 342**	**$ 354**	**$ 367**	**$ 380**	**$ 393**	**$ 407**	**$ 421**	**$ 218**	**$ 113**	**$ 58**	**$ 30**	**$ 16**	**$ 8**	**$ 4**
% of WW sales								*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*
WW sales (M)	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ -**	**$ 37**	**$ 118**	**$ 289**	**$ 605**	**$ 912**	**$ 1,173**	**$ 1,342**	**$ 1,521**	**$ 1,712**	**$ 1,772**	**$ 1,834**	**$ 1,899**	**$ 1,965**	**$ 2,034**	**$ 2,106**	**$ 1,090**	**$ 564**	**$ 292**	**$ 151**	**$ 78**	**$ 40**	**$ 21**
% y/y									*145%*	*109%*	*51%*	*29%*	*14%*	*13%*	*13%*	*4%*	*4%*	*4%*	*4%*	*4%*	*4%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*	*-48%*

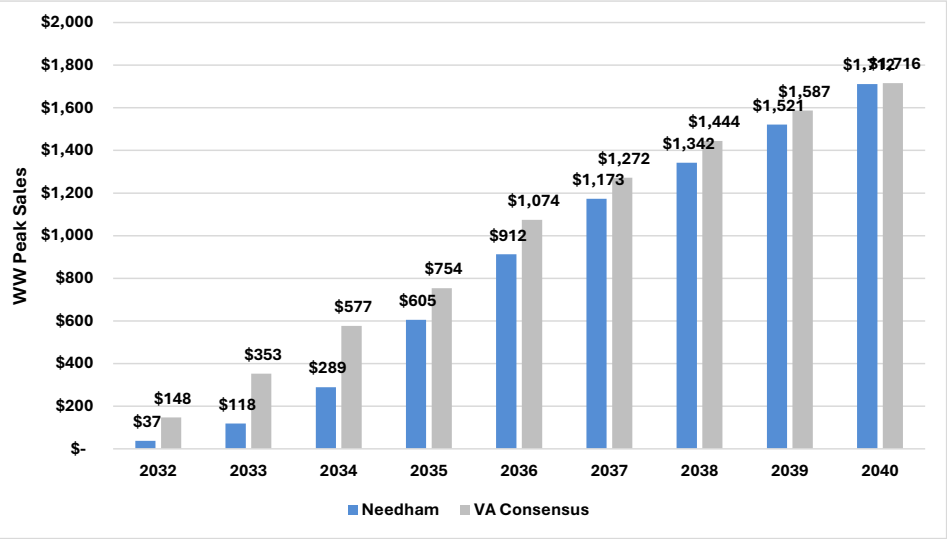
Source: Needham & Co.

CMP-002 WW Revenue Estimates in SYNGAP1 - Needham



Source: Needham & Co.

CMP-002 WW Revenue Estimates in SYNGAP1 - Needham vs. VA Consensus



Source: Needham & Co.

CMP-002 in SYNGAP1: Peak Sales Summary

CMP-002 in SYNGAP1: Peak Sales Summary		
WAC annual price (2032 launch)	$	575,000
% annual price increase		*3%*
WAC annual price (2046 peak)	$	869,739
GTN discount		*15%*
adherence/persistence		*75%*
Net annual price (2046 peak)	$	554,459
Total SYNGAP1 patients (US)		12,154
% diagnosed		*50%*
% penetration CMP-002		*50%*
Total CMP-002 patients treated (US)		3,039
US peak sales (2046)	**$**	**1,685**
ROW multiplier		*0.25x*
ROW peak sales (2046)	**$**	**421**
WW peak sales (2046)	**$**	**2,106**

Source: Needham & Co.

CMP-002 Peak U.S. Revenues in SYNGAP1 Variance Analysis

CMP-002 Peak U.S. Revenue in SYNGAP1 - Variance Analysis

Peak prevalence: **12,154**

Peak net price: **$ 538,309**

Diagnosed & Addressable \ CMP-002 peak penetration	30%	40%	50%	60%	70%
30%	$ 589	$ 785	$ 981	$ 1,178	$ 1,374
40%	$ 785	$ 1,047	$ 1,309	$ 1,570	$ 1,832
50%	$ 981	$ 1,309	$ 1,636	$ 1,963	$ 2,290
60%	$ 1,178	$ 1,570	$ 1,963	$ 2,355	$ 2,748
70%	$ 1,374	$ 1,832	$ 2,290	$ 2,748	$ 3,206

Source: Needham & Co.

VALUATION

CAMP Valuation - DCF Model (Needham & Co.)

Company: CAMP4 Therapeutics Ticker: CAMP																											
Valuation date	10/13/26																										
YE date	12/31/26																										
Discount Rate	9.0%																										
Terminal Growth Rate	0%																										
Probability	2026	2027	2028	2029	2030	2031	2032	2033	2034	2035	2036	2037	2038	2039	2040	2041	2042	2043	2044	2045	2046	2047	2048	2049	2050	Terminal
CMP-002 in SYNGAP1 25%	0	0	0	0	0	0	9	30	72	151	228	293	335	380	428	443	459	475	491	509	526	272	141	73	38	
Collaboration revenue 100%	3	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	
Total revenue	3	0	0	0	0	0	9	30	72	151	228	293	335	380	428	443	459	475	491	509	526	272	141	73	38	
% y/y									*145%*	*109%*	*51%*	*29%*	*14%*	*13%*	*13%*	*4%*	*4%*	*4%*	*4%*	*4%*	*4%*	*-48%*	*-48%*	*-48%*	*-48%*	
COGS	0	0	0	0	0	0	1	2	6	12	18	23	27	30	34	35	37	38	39	41	42	22	11	6	3	
COGS %	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	*8%*	
R&D	45	14	16	23	35	42	11	6	4	2	2	3	3	4	4	4	5	5	5	5	5	3	1	1	0	
R&D as a % of revenue	*1331%*	-	-	-	-	-	*120%*	*20%*	*5%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	*1%*	
SG&A	21	6	7	8	10	19	23	30	47	74	89	100	107	110	111	111	110	109	108	107	105	54	28	15	8	
SG&A as a % of sales	*625%*	-	-	-	-	-	*250%*	*100%*	*65%*	*49%*	*39%*	*34%*	*32%*	*29%*	*26%*	*25%*	*24%*	*23%*	*22%*	*21%*	*20%*	*20%*	*20%*	*20%*	*20%*	
risk-adjusted EBIT ($M)	(63)	(20)	(23)	(31)	(45)	(61)	(26)	(8)	16	64	119	167	198	236	278	292	307	323	339	356	374	193	100	52	27	
% NOL/Tax asset carryforward	*80%*	*80%*	*80%*	*80%*	*80%*	*80%*	*80%*	*80%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	*100%*	
NOLs/Tax Assets ($M)	(280)	(296)	(314)	(339)	(375)	(423)	(444)	(451)	(435)	(371)	(253)	(85)														
Tax rate	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	*20%*	
Taxes paid ($M)	0	0	0	0	0	0	0	0	0	0	0	0	40	47	56	58	61	65	68	71	75	39	20	10	5	
risk-adjusted EBIAT ($M)	(63)	(20)	(23)	(31)	(45)	(61)	(26)	(8)	16	64	119	167	158	189	223	234	246	258	271	285	299	155	80	41	21	
D&A	2	2	1	-	-	-	-	-	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	
CapEx	(0)	(0)	(0)	-	-	-	-	-	(0)	(0)	(0)	(1)	(1)	(1)	(1)	(2)	(2)	(2)	(2)	(2)	(2)	(2)	(2)	(1)	(1)	
Δ Non-cash Working Capital	(17)	0	(1)	0	(2)	(6)	(7)	(2)	1	4	2	1	1	0	(0)	(1)	(1)	(1)	(1)	(1)	(1)	5	2	1	1	
risk-adjusted FCF ($M)	(78)	(18)	(23)	(30)	(47)	(67)	(33)	(10)	17	67	120	168	158	188	221	232	244	256	269	282	296	158	81	42	21	238
Years	0.22	1.22	2.22	3.22	4.22	5.22	6.22	7.22	8.22	9.22	10.22	11.22	12.22	13.22	14.22	15.22	16.22	17.22	18.22	19.22	20.22	21.22	22.22	23.22	24.22	24.22
Discount Factor	0.98	0.90	0.83	0.76	0.70	0.64	0.59	0.54	0.49	0.45	0.41	0.38	0.35	0.32	0.29	0.27	0.25	0.23	0.21	0.19	0.18	0.16	0.15	0.14	0.12	0.12
NPV FCF ($M)	**(77)**	**(16)**	**(19)**	**(23)**	**(33)**	**(43)**	**(19)**	**(5)**	**9**	**30**	**50**	**64**	**55**	**60**	**65**	**62**	**60**	**58**	**56**	**54**	**52**	**25**	**12**	**6**	**3**	**30**

NPV cash flows (M)	$	485
PV of Terminal Value (M)	$	30
Terminal Value as a % of Total		6%
Enterprise Value (M)	**$**	**514**
Cash (M)	$	135
Debt (M)	$	-
Net Cash (M)	$	135
Equity Value (M)	**$**	**649**
Fully Diluted S/O (M)		98
Price/Share	**$**	**7**

NPV Per Share Analysis

		(M)	NPV/share
CMP-002 in SYNGAP1	$	514	$ 5
Net cash	$	135	$ 1
Total	**$**	**649**	**$ 7**

Sensitivity Analysis

Terminal Growth Rate \ Discount Rate	7.0%	8.0%	9.0%	10.0%	11.0%
-50%	$ 9	$ 7	$ 6	$ 6	$ 5
-25%	$ 9	$ 7	$ 6	$ 6	$ 5
-10%	$ 9	$ 8	$ 6	$ 6	$ 5
0%	$ 9	$ 8	**$ 7**	$ 6	$ 5
1%	$ 9	$ 8	$ 7	$ 6	$ 5
2%	$ 9	$ 8	$ 7	$ 6	$ 5
3%	$ 9	$ 8	$ 7	$ 6	$ 5

Source: Needham & Co.

CAMP Valuation - sum-of-the-parts revenue multiple analysis (Needham & Co.)

EV/Peak Revenue Valuation	CMP-002 in SYNGAP1
Base year	2026
Peak sales year	2043
Revenue multiple	7.0x
WACC (discount rate)	9.0%
Peak revenue non-risk-adjusted (M)	$ 1,843
Probability of success	25%
Peak revenue risk-adjusted (M)	$ 461
Years	17.0
Value (M)	$ 745
Fully Diluted S/O (M)	98
Value/share	**$ 8**

EV/Peak Revenue Valuation Summary	Value (M)	Per Share	%
CMP-002 in SYNGAP1	$ 745	$ 8	85%
Platform/pipeline	$ -	$ -	0%
Net cash	$ 135	$ 1	15%
Fully Diluted S/O (M)	98		
Enterprise Value (M)	**$ 745**	**-**	
Equity Value (M)	**$ 880**	**$ 9**	**100%**

Source: Needham & Co.

VALUATION SUMMARY

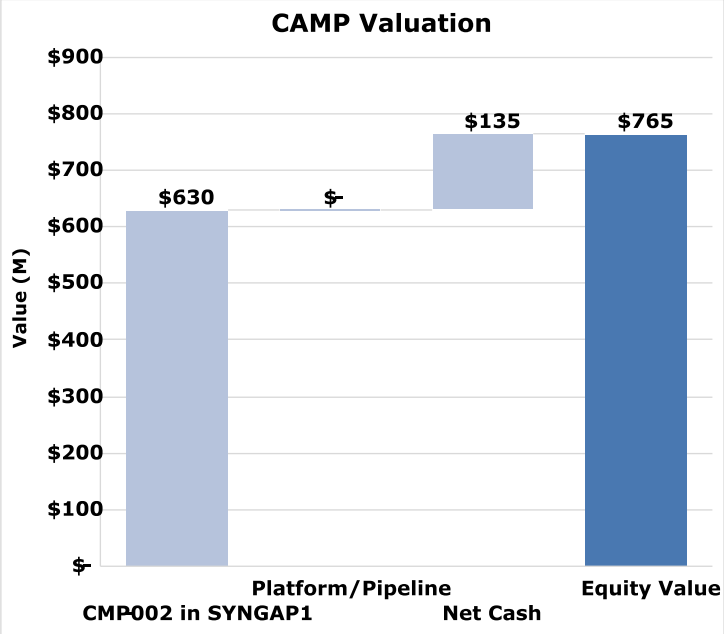
CAMP Valuation Summary

Valuation	Equity Value (M)	Price/Share	Weight
DCF	$ 649	$ 7	50%
EV/Peak Revenue(x)	$ 880	$ 9	50%
Weighted Average	**$ 765**	**$ 8**	

Valuation - SOTP	Value (M)	$/Share
CMP-002 in SYNGAP1	$ 630	$ 6
Platform/Pipeline	$ -	$ -
Net Cash	$ 135	$ 1
Equity Value	**$ 765**	**$ 8**
Fully Diluted S/O (M)	98	98

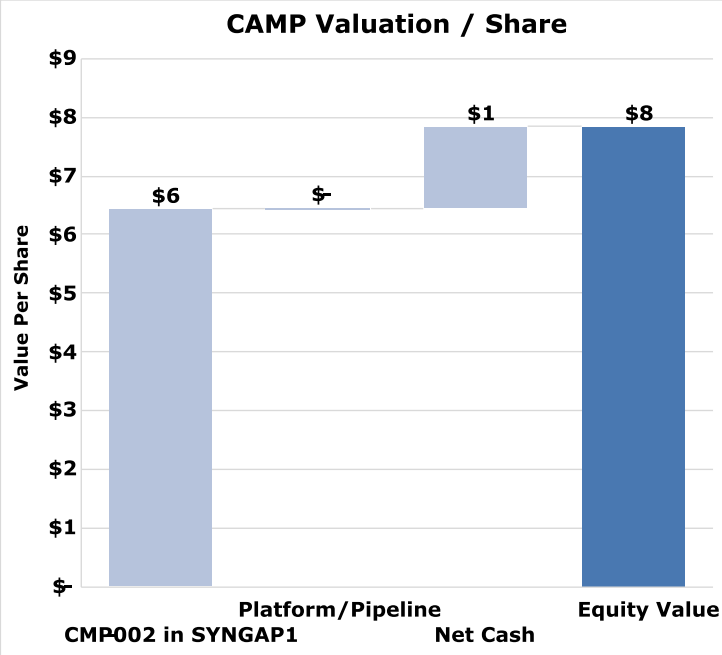
Source: Needham & Co.

CAMP Valuation



Source: Needham & Co.

CAMP Valuation Per Share



Source: Needham & Co.

Income Statement

CAMP4 Therapeutics (CAMP) Income Statement ($M)																	
FY: December	2024A	2025A	1Q26A	2Q26A	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E
Product Revenue																	
CMP-002 [SYNGAP1]	-	-	-	-	-	-	-	-	-	-	-	-	37	118	289	605	912
Total product revenue	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**-**	**37**	**118**	**289**	**605**	**912**
%y/y														*216%*	*145%*	*109%*	*51%*
Research & collaboration revenue	0.7	3.5	1.3	0.7	0.7	0.7	3.4	-	-	-	-	-	-	-	-	-	-
Total Revenue	**0.7**	**3.5**	**1.3**	**0.7**	**0.7**	**0.7**	**3.4**	**-**	**-**	**-**	**-**	**-**	**37**	**118**	**289**	**605**	**912**
COGS (incl. royalties)	-	-	-	-	-	-	-	-	-	-	-	-	3.0	9.5	23.1	48.4	73
COGS %	-	-	-	-	-	-	-	-	-	-	-	-	*8%*	*8%*	*8%*	*8%*	*8%*
Gross Income	**1**	**3.5**	**1.3**	**0.7**	**0.7**	**0.7**	**3.4**	**-**	**-**	**-**	**-**	**-**	**34**	**109**	**266**	**556**	**839**
Gross margin														*92%*	*92%*	*92%*	*92%*
GAAP R&D	38.8	38.2	10.2	11.0	12.0	12.0	45.2	55.0	65.0	90.0	139.5	167.4	44.9	23.7	14.5	6.0	9
GAAP SG&A	14.9	17.4	4.2	5.0	6.0	6.0	21.2	24.0	26.0	32.0	40.0	75.0	93.5	118.3	188.1	296.4	356
Impairment of right-of-use asset	-	0.5	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total GAAP OpEx	53.7	56.1	14.4	16.0	18.0	18.0	66.4	79.0	91.0	122	180	242	138	142	203	302	365
% y/y		*4%*	*3%*	*10%*	*25%*	*37%*	*18%*	*19%*	*15%*	*34%*	*47%*	*35%*	*-43%*	*3%*	*43%*	*49%*	*21%*
% of revenue													*370%*	*120%*	*70%*	*50%*	*40%*
Operating Income	**(53.1)**	**(52.6)**	**(13.1)**	**(15.3)**	**(17.3)**	**(17.3)**	**(63.0)**	**(79.0)**	**(91.0)**	**(122)**	**(180)**	**(242)**	**(104)**	**(33)**	**64**	**254**	**474**
Operating Margin																*42%*	*52%*
Interest income (expense)	1.3	2	0.9	-	-	-	0.9	2.0	3.6	5.0	8.8	11.5	13.2	14.4	14.0	15.7	21
Change in FV derivative tranche liability	-	(30)	(6.2)	-	-	-	(6.2)	-	-	-	-	-	-	-	-	-	-
Other income (expense)	(0.0)	(0)	0.0	-	-	-	0.0	-	-	-	-	-	-	-	-	-	-
Pre-tax Income	**(51.8)**	**(80.4)**	**(18.3)**	**(15.3)**	**(17.3)**	**(17.3)**	**(68.2)**	**(77.0)**	**(87.4)**	**(117)**	**(171)**	**(231)**	**(91)**	**(19)**	**78**	**270**	**496**
Income taxes	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Tax rate %	-	-	-	-	-	-	-	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*	*0%*
Net Income (loss)	**(51.8)**	**(80.4)**	**(18.3)**	**(15.3)**	**(17.3)**	**(17.3)**	**(68.2)**	**(77.0)**	**(87.4)**	**(117)**	**(171)**	**(231)**	**(91)**	**(19)**	**78**	**270**	**496**
GAAP Diluted EPS	**(11.04)**	**(2.65)**	**(0.32)**	**(0.26)**	**(0.28)**	**(0.28)**	**(1.13)**	**(1.04)**	**(1.03)**	**(1.11)**	**(1.36)**	**(1.59)**	**(0.59)**	**(0.12)**	**0.40**	**1.37**	**2.49**
Shares - Diluted [M]	4.7	30.4	57.9	58.2	62.5	62.7	60.3	74.2	85.2	105.8	125.8	145.2	154.5	155.3	195.9	197.4	199.1
Needham & Company, LLC																	

Source: Company documents; Needham & Co.

Balance Sheet

CAMP4 Therapeutics (CAMP)													
Balance Sheet ($M)													
FY: December	2024A	2025A	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E
Current assets:													
Cash and cash equivalents	64	110	34	115	186	381	526	613	678	672	771	1,074	1,607
Restricted cash, current	-	1	-	-	-	-	-	-	-	-	-	-	-
Prepaid expenses and other current assets	2	3	4	5	7	9	12	16	19	21	22	22	22
Total current assets	66	114	38	121	193	390	538	628	697	693	793	1,096	1,630
Noncurrent assets:													
Restricted cash, noncurrent	2	1	-	-	-	-	-	-	-	-	-	-	-
Property & equipment	4	2	2	2	2	2	2	2	2	2	2	3	4
Operating lease right-of-use assets	6	0	0	0	0	0	0	0	0	0	0	0	0
Other assets	0	0	0	0	0	0	0	0	0	0	0	0	0
Total Assets	**78**	**118**	**40**	**123**	**195**	**393**	**540**	**631**	**699**	**695**	**795**	**1,099**	**1,634**
Current liabilities:													
Accounts payable	1	0	1	2	2	2	4	5	3	3	4	6	7
Accrued expenses and other current liabilities	5	6	7	8	8	10	9	5	3	3	4	6	7
Current portion of deferred revenue	0	9	-	-	-	-	-	-	-	-	-	-	-
Operating lease liabilities, current	3	0	-	-	-	-	-	-	-	-	-	-	-
Other current liabilities	0	0	-	-	-	-	-	-	-	-	-	-	-
Total current liabilities	10	16	8	9	10	12	13	10	6	6	8	12	15
Noncurrent liabilities:													
Deferred revenue	-	8	-	-	-	-	-	-	-	-	-	-	-
Operating lease liabilities	5	2	2	2	2	2	2	2	2	2	2	2	2
Derivative tranche liability	-	45	-	-	-	-	-	-	-	-	-	-	-
Other liabilities	0	0	0	0	0	0	0	0	0	0	0	0	0
Total liabilities	**15**	**70**	**10**	**11**	**12**	**14**	**14**	**11**	**7**	**7**	**10**	**14**	**16**
Common stock	0	0	0	0	0	0	0	0	0	0	0	0	0
Additional paid-in capital	275	340	391	549	708	1,020	1,338	1,663	1,826	1,841	1,861	1,891	1,928
Accumulated gain (deficit)	(212)	(292)	(360)	(437)	(525)	(642)	(813)	(1,043)	(1,134)	(1,153)	(1,075)	(806)	(310)
Total stockholders' equity	**63**	**48**	**31**	**112**	**183**	**379**	**526**	**619**	**692**	**688**	**786**	**1,086**	**1,618**
Total liabilities & stockholders' equity	**78**	**118**	**40**	**123**	**195**	**393**	**540**	**631**	**699**	**695**	**795**	**1,099**	**1,634**
Balance Check	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*	*OK*
Balance Sheet Drivers													
Prepaid expenses/Other current assets (% of revenue)	*360%*	*92%*	*124%*	*nm*	*nm*	*nm*	*nm*	*nm*	*50%*	*17%*	*7%*	*4%*	*2%*
Prepaid expenses/Other current assets (% y/y)	*nm*	*38%*	*30%*	*30%*	*30%*	*30%*	*30%*	*30%*	*20%*	*10%*	*5%*	*2%*	*2%*
Accounts payable (% of OpEx)	*2%*	*0%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*
Accrued expenses/liabilities (% of OpEx)	*9%*	*10%*	*10%*	*10%*	*9%*	*8%*	*5%*	*2%*	*2%*	*2%*	*2%*	*2%*	*2%*
Needham & Company, LLC													

Source: Company documents; Needham & Co.

Cash Flow Statement

CAMP4 Therapeutics (CAMP)													
Cash Flow Statement ($M)													
FY: December	2024A	2025A	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E	2036E
Cash Flows from Operating Activities													
Net income (loss)	(52)	(80)	(68)	(77)	(87)	(117)	(171)	(231)	(91)	(19)	78	270	496
Adjustments:													
Depreciation and amortization	2	2	1	-	-	-	-	-	0	0	0	0	0
Stock-based compensation expense	4	4	5	8	9	12	18	24	14	14	20	30	36
Change in fair value of derivative tranche liability	-	30	(45)	-	-	-	-	-	-	-	-	-	-
Impairment of right-of-use asset	-	0	-	-	-	-	-	-	-	-	-	-	-
Loss on the sale of property and equipment	-	0	-	-	-	-	-	-	-	-	-	-	-
Non-cash gain on lease modification	-	(0)	-	-	-	-	-	-	-	-	-	-	-
Non-cash lease expense	2	2	-	-	-	-	-	-	-	-	-	-	-
Non-cash interest expense	0	0	-	-	-	-	-	-	-	-	-	-	-
Changes in operating assets and liabilities:													
Prepaid expenses and other current assets	(1)	(1)	(1)	(1)	(2)	(2)	(3)	(4)	(3)	(2)	(1)	(0)	(0)
Accounts payable	0	(1)	1	0	0	1	1	1	(2)	0	1	2	1
Accrued expenses and other current liabilities	2	1	1	1	0	2	(1)	(4)	(2)	0	1	2	1
Deferred revenue	0	17	(18)	-	-	-	-	-	-	-	-	-	-
Operating lease assets and liabilities	(3)	(3)	(0)	-	-	-	-	-	-	-	-	-	-
Net cash provided (used) in Operating Activities	**(46)**	**(30)**	**(124)**	**(69)**	**(79)**	**(105)**	**(155)**	**(213)**	**(84)**	**(6)**	**99**	**304**	**534**
Cash Flows from Investing Activities													
Purchase of property and equipment	(0)	(0)	(0)	-	-	-	-	-	(0)	(0)	(0)	(1)	(1)
Proceeds from the sale of property and equipment	-	0	-	-	-	-	-	-	-	-	-	-	-
Net cash provided (used) in Investing Activities	**(0)**	**(0)**	**(0)**	**0**	**0**	**0**	**0**	**0**	**(0)**	**(0)**	**(0)**	**(1)**	**(1)**
Cash flows from Financing Activities													
Proceeds from issuance of common stock, net	**76**	**58**	**47**	**150**	**150**	**300**	**300**	**300**	**150**	**-**	**-**	**-**	**-**
Proceeds from issuance of pre-funded warrants, net	-	4	-	-	-	-	-	-	-	-	-	-	-
Proceeds from exercise of common stock options	0	0	-	-	-	-	-	-	-	-	-	-	-
Proceeds allocated to the derivative tranche liability, net	-	15	-	-	-	-	-	-	-	-	-	-	-
Principal payments on financing obligation	(0)	(0)	-	-	-	-	-	-	-	-	-	-	-
Principal payments on finance leases	(0)	(0)	-	-	-	-	-	-	-	-	-	-	-
Payments of deferred issuance costs	(4)	-	-	-	-	-	-	-	-	-	-	-	-
Net cash provided (used) in Financing Activities	**72**	**76**	**47**	**150**	**150**	**300**	**300**	**300**	**150**	**0**	**0**	**0**	**0**
Net increase (decrease) in cash & equivalents and restricted cash	**26**	**46**	**(78)**	**81**	**71**	**195**	**145**	**87**	**66**	**(6)**	**99**	**303**	**533**
Beginning of period	40	66	112	34	115	186	381	526	613	678	672	771	1,074
End of period	**66**	**112**	**34**	**115**	**186**	**381**	**526**	**613**	**678**	**672**	**771**	**1,074**	**1,607**
Cash Flow Statement Drivers													
D&A (% of PP&E)	*248%*	*447%*	*400%*	*300%*	*200%*	*100%*	*50%*	*20%*	*10%*	*5%*	*5%*	*5%*	*5%*
Stock-based compensation (% of OpEx)	*8%*	*29%*	*7%*	*10%*	*10%*	*10%*	*10%*	*10%*	*10%*	*10%*	*10%*	*10%*	*10%*
P&E (% of revenue)	*68%*	*8%*	*5%*	*2%*	*2%*	*2%*	*1%*	*1%*	*0%*	*0%*	*0%*	*0%*	*0%*
P&E (% y/y)	*nm*	*-37%*	*-39%*	*-100%*	*nm*	*nm*	*nm*	*nm*	*nm*	*216%*	*145%*	*109%*	*51%*
Needham & Company, LLC													

Source: Company documents; Needham & Co.

VALUATION (PRICE TARGET: $8.00)

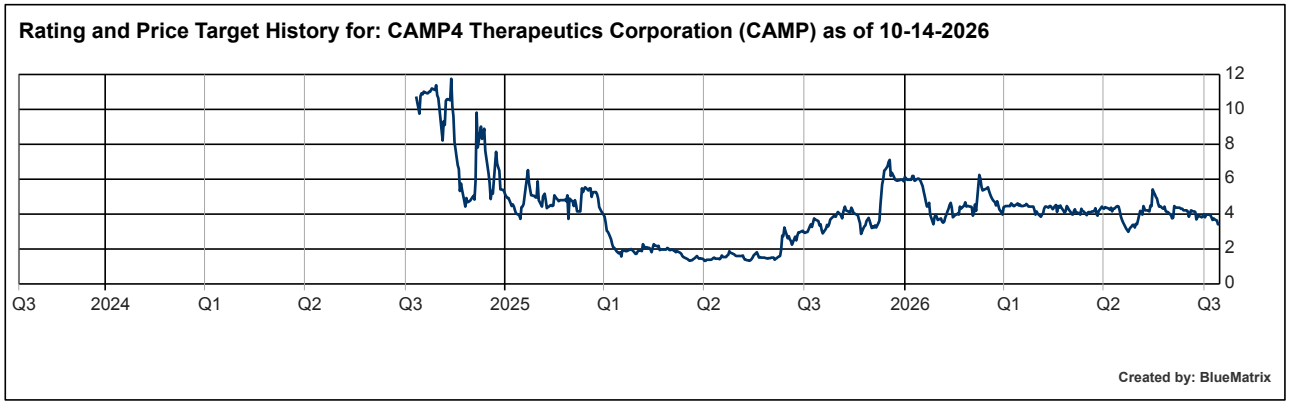
Our valuation and price target are based on a weighted average of two valuation methods: 1) a DCF analysis through 2050 using risk-adjusted revenues, a 9% WACC, and 0% terminal growth; and 2) a sum-of-the-parts EV/peak revenue multiple analysis using a 7.0x peak revenue multiple and risk-adjusted revenues.

RISKS TO TARGET

CAMP

Investment risks include, but are not limited to, clinical development, competitive, regulatory, commercial, financial, and legal risks associated with CAMP4 products.

- **Clinical Development.** CAMP4 may be unsuccessful in progressing its preclinical assets into clinical studies.
- **Competitive.** Drug development is competitive, and there is no guarantee that CAMP4 will obtain commercial success with its drugs, if approved.
- **Regulatory:** Drugs require regulatory approval prior to launch. No drug from CAMP4 has been approved by a regulatory authority.
- **Commercial:** CAMP4 will need to sufficiently differentiate its clinical candidates from other drugs currently on the market and in development. The company will also need to build a commercial infrastructure to compete with companies with substantially more resources.
- **Financial**. We do not anticipate profitability for several years. Clinical development of CAMP4's drug candidates involves relatively long timelines and significant financial resources. We expect CAMP4 will require additional funds as it takes its clinical candidates through clinical trials. CAMP4 may need to raise additional capital, and there is no guarantee that the company will be able to obtain financing on favorable terms.
- **Legal:** CAMP4 owns intellectual property for its assets, including CMP-002. However, there is no guarantee that any pending patent applications will issue or that the company will achieve adequate protection of its product candidates. There are risks that CAMP4 products may infringe patents assigned to other entities.



ANALYST CERTIFICATION

I, Joseph Stringer hereby certify that the views expressed in this research report accurately reflect my personal views about the subject company (ies) and its (their) securities. I, also certify that I, have not been, and will not be receiving direct or indirect compensation in exchange for expressing the specific recommendation(s) in this report.

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	% of companies under coverage with this rating	% for which investment banking services have been provided for in the past 12 months
Buy	80	20
Hold	20	3
Underperform	< 1	0
Rating Suspended	0	0
Restricted	< 1	0

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DISCLOSURES

The research analyst and research associate have received compensation based upon various factors, including quality of research, investor client feedback, and the Firm's overall revenues, which includes investment banking revenues for the following: CAMP4 Therapeutics Corporation

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